FlexiCT: 20-case error review

CT images, model Grad-CAM and radiologist interpretations

Final-block fine-tuning | five-fold CV ensemble | complete development set

The original held-out cohort contained 278 patients. These 20 cases were selected because the ensemble prediction disagreed with the recorded class. All original labels and all cases remain in reported performance calculations.

How to read this report

Each case has an interpretation page, the existing three-plane CT/Grad-CAM panel (Figure S10), and the existing CT overview (Figure S11). Radiologist summaries and analyst interpretations are reproduced from publication Table S12. Diagnostic values are reproduced from Table S14. The published figure pages are appended without re-rendering or inference.

Interpretation boundaries

The reread is exploratory and not blinded adjudication. R20 retains a provisional linkage. A 3D transformer-token Grad-CAM adaptation targets class probability; slices pass through the predicted-map maximum, not a verified lesion. Heatmaps do not establish clinical localization, causality or correctness. No lesion annotations were supplied.

Fourteen of 20 cases had a positive top-mask probability drop exceeding the random-mask mean. All 20 exceeded the random mean relatively; six had non-positive drops. This distinction is preserved and is not an attribution-validation claim.

Sources

Publication_Results_4.ipynb; supplementary Tables S12-S14; Figures S10 and S11. Internal identifier linkage, audit outputs and verbatim correspondence are excluded. This compilation uses existing published outputs; it is not an additional experiment.

R01 | Reference-label discrepancy

Recorded class: Normal | FlexiCT prediction: Appendicitis

Radiologist interpretation

Clear appendicitis on reread; supports the model class.

Interpretation of the reread

Reconsider recorded Normal label

Prediction and explanation diagnostics

Predicted-class probability	0.9994
Recorded-class probability	0.0001
Fold votes for ensemble prediction	5
Top-mask probability drop	0.9873
Random-mask mean probability drop	0.0303
Positive drop exceeds random	True
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

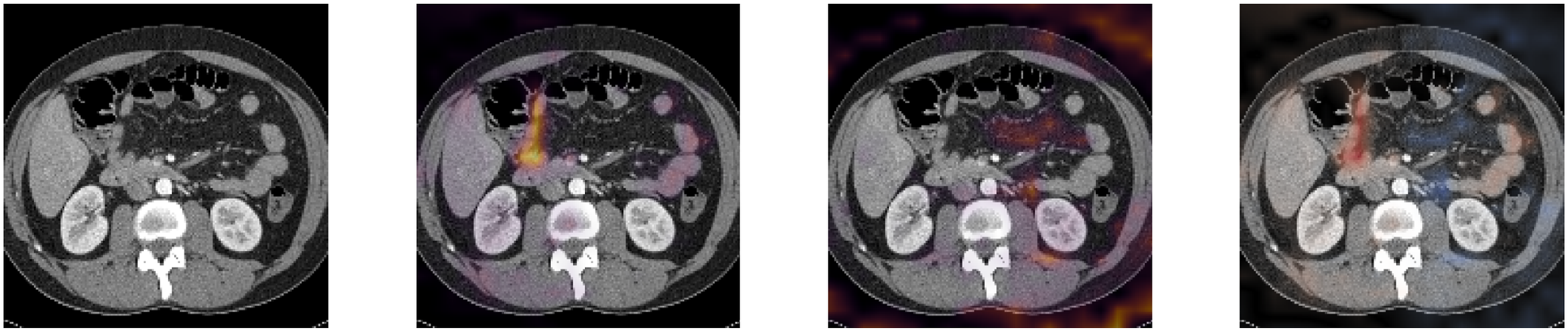
R01 · Recorded: Normal · Predicted: Appendicitis
p(predicted)=0.999 · p(recorded)=0.000 · Reference-label discrepancy

CT · WL 40 / WW 400
Axial · model index 109

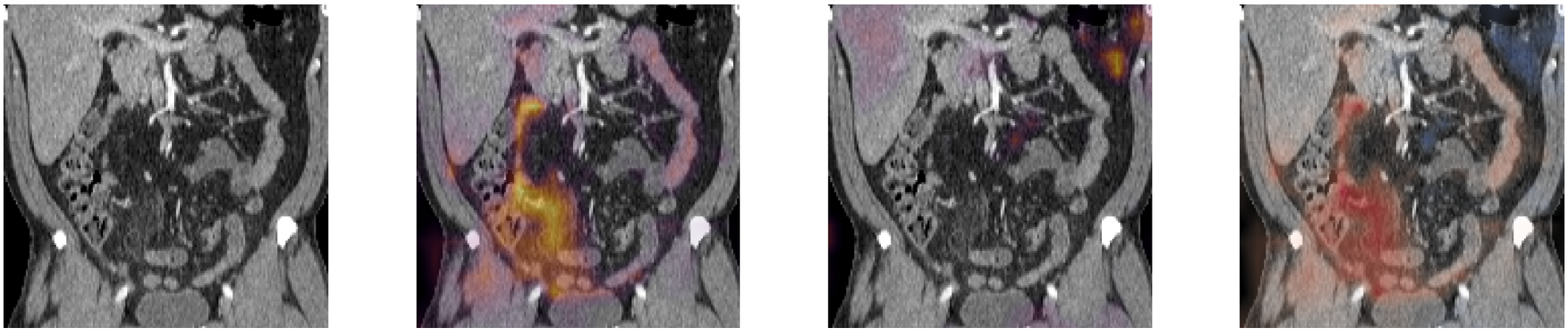
Predicted-class CAM

Recorded-class CAM

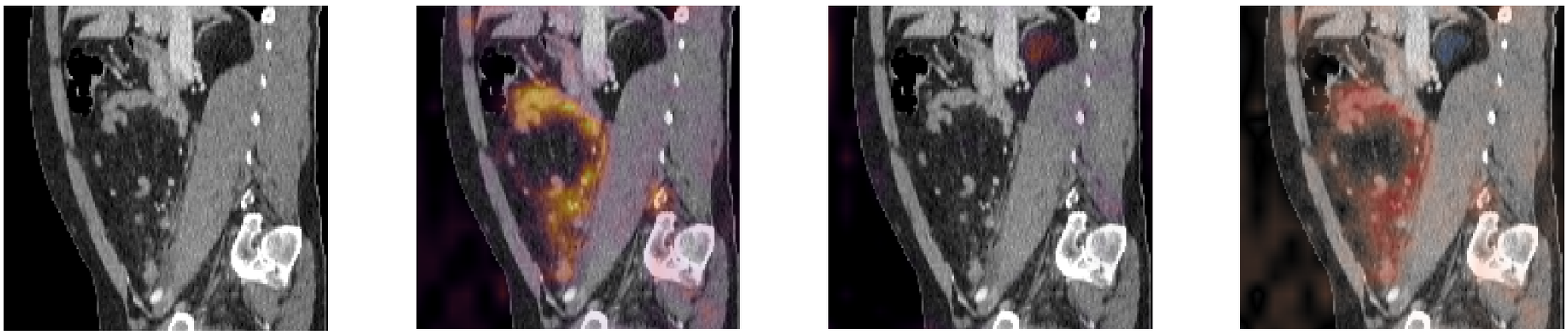
Signed CAM contrast



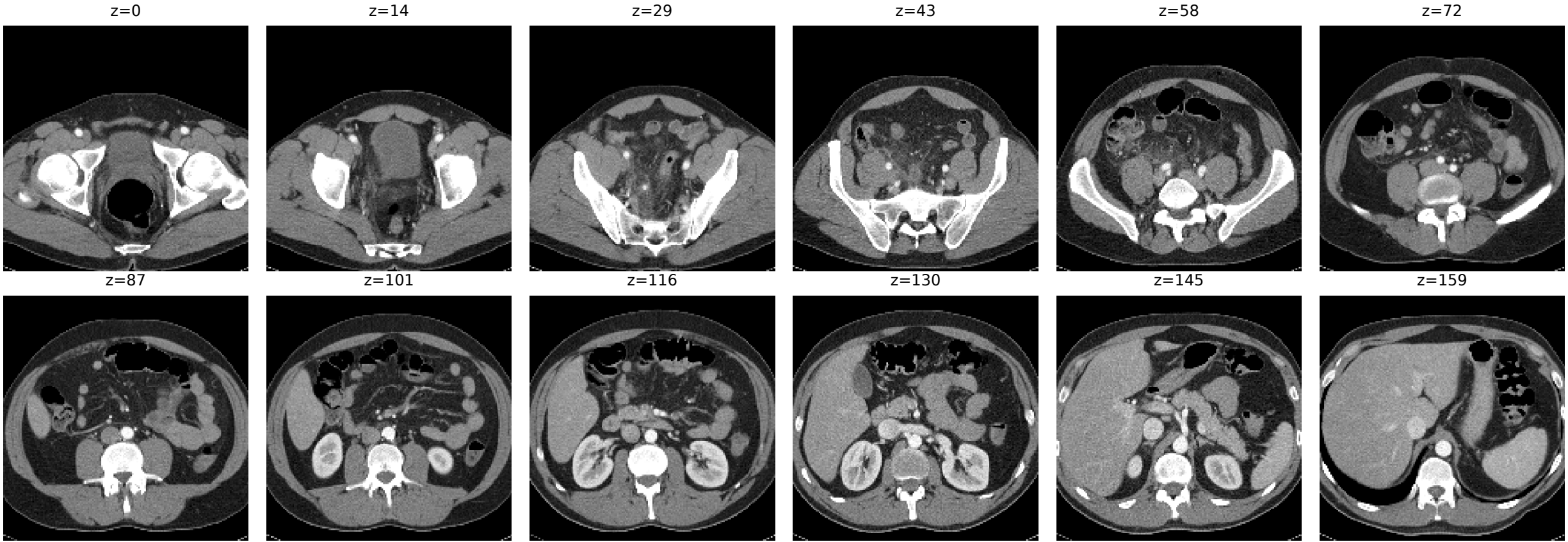
Coronal · model index 75



Sagittal · model index 58



LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R01: model-input axial overview | Normal → Appendicitis
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R02 | Other disease / proposed exclusion

Recorded class: Normal | FlexiCT prediction: Sigmoid

Radiologist interpretation

Colitis and jejunitis; neither normal nor appendicitis nor sigmoid diverticulitis.

Interpretation of the reread

Outside the five mutually exclusive labels

Prediction and explanation diagnostics

Predicted-class probability	0.9952
Recorded-class probability	0.0041
Fold votes for ensemble prediction	5
Top-mask probability drop	0.9945
Random-mask mean probability drop	0.3411
Positive drop exceeds random	True
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

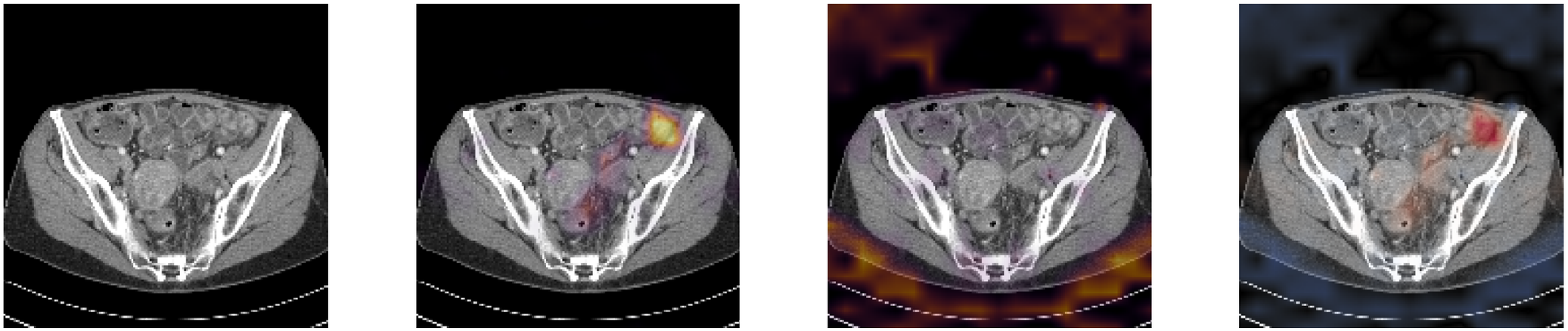
R02 · Recorded: Normal · Predicted: Sigmoid
p(predicted)=0.995 · p(recorded)=0.004 · Other disease / proposed exclusion

CT · WL 40 / WW 400
Axial · model index 42

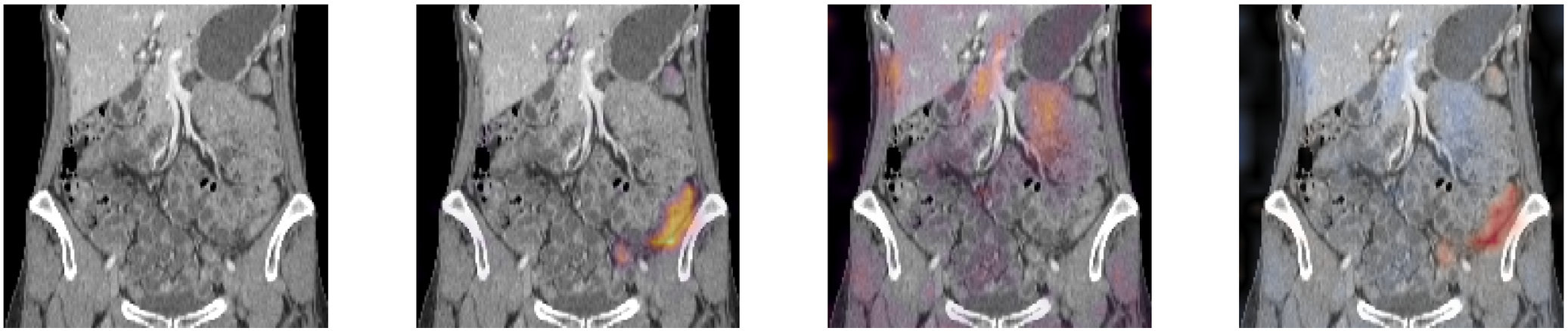
Predicted-class CAM

Recorded-class CAM

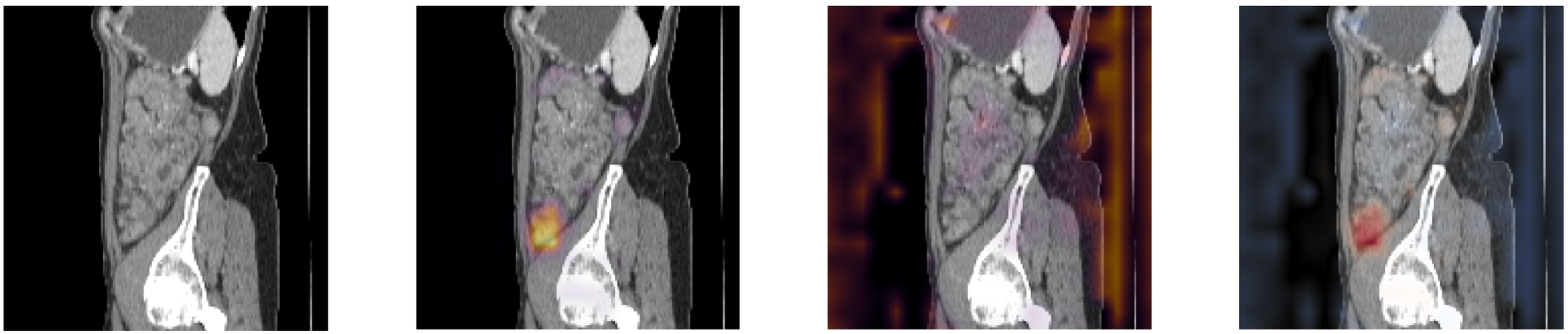
Signed CAM contrast



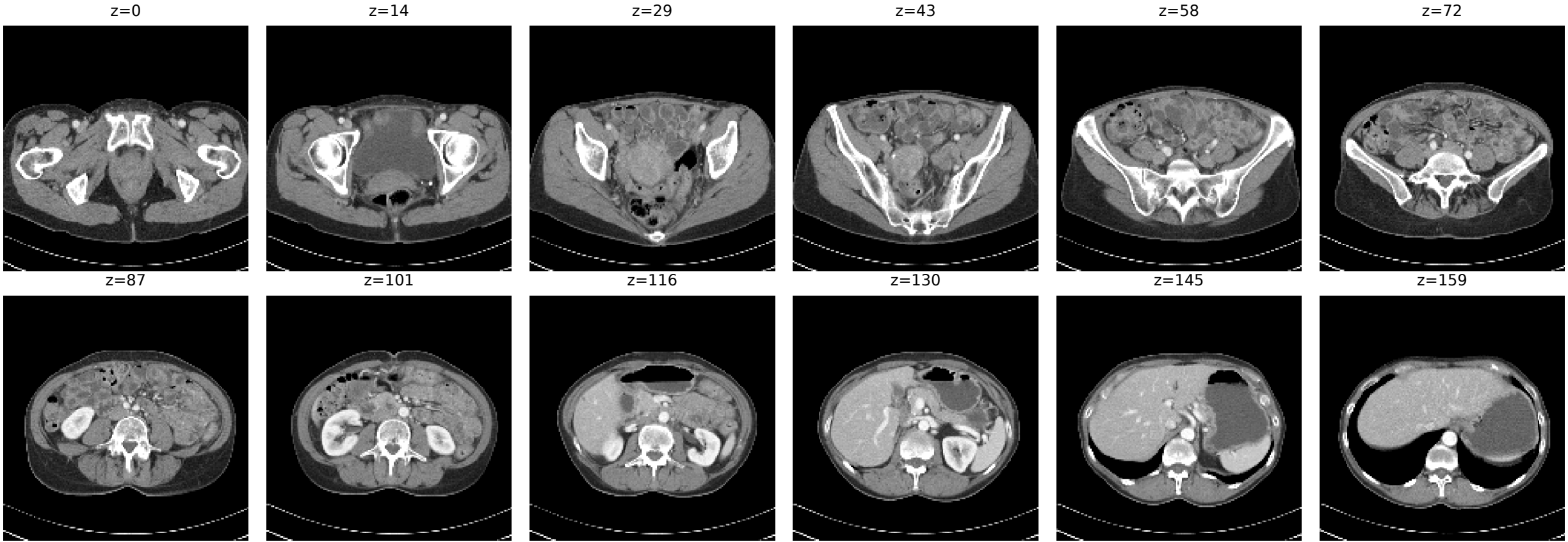
Coronal · model index 67



Sagittal · model index 125



LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R02: model-input axial overview | Normal → Sigmoid
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R03 | Other disease / proposed exclusion

Recorded class: Normal | FlexiCT prediction: Cholezystitis

Radiologist interpretation

Gallbladder-adjacent enhancing lesion ultimately considered a liver tumor mimicking inflammation.

Interpretation of the reread

Reviewer recommends exclusion

Prediction and explanation diagnostics

Predicted-class probability	0.9923
Recorded-class probability	0.0074
Fold votes for ensemble prediction	5
Top-mask probability drop	0.9910
Random-mask mean probability drop	0.0249
Positive drop exceeds random	True
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

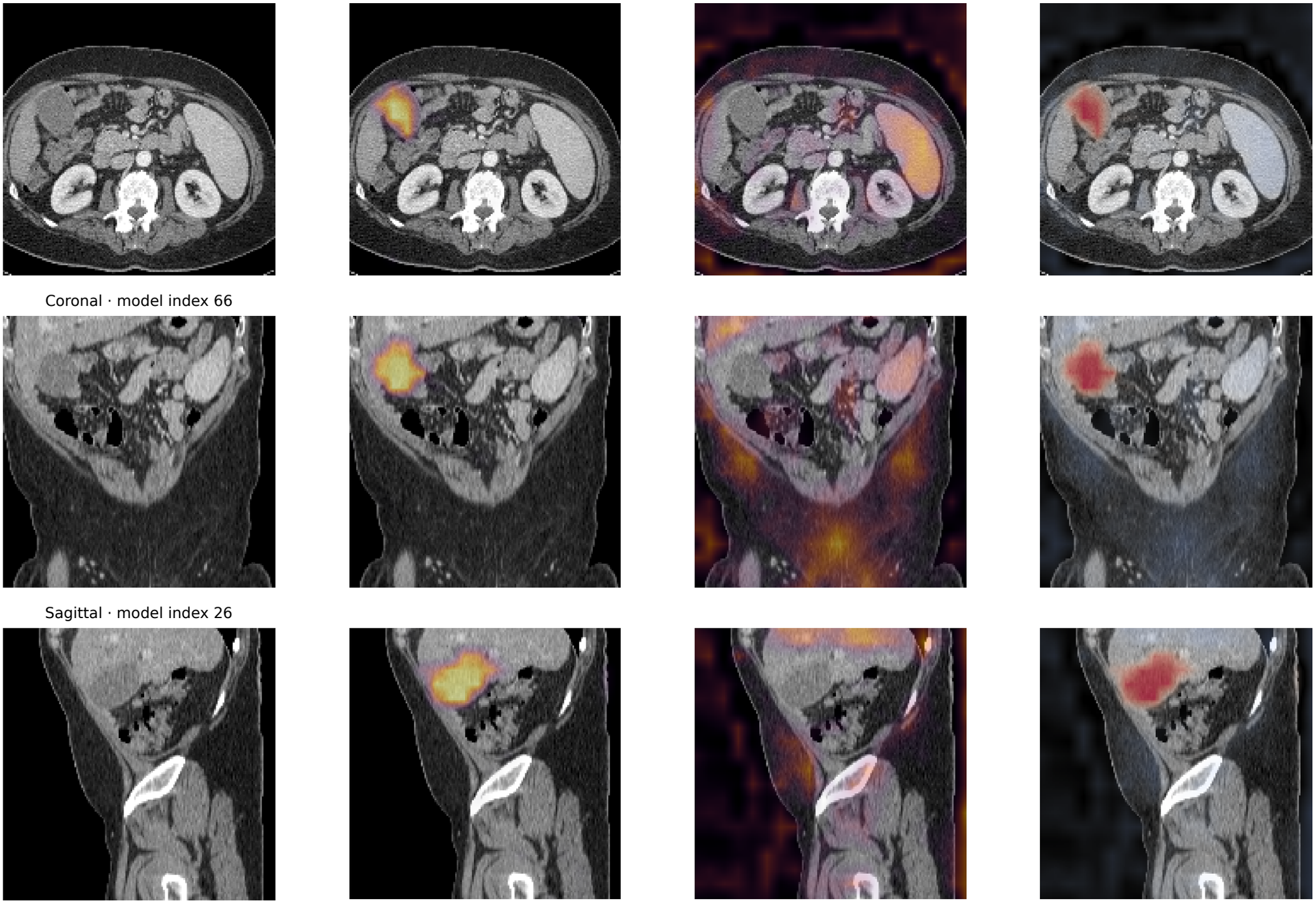
R03 · Recorded: Normal · Predicted: Cholezystitis
p(predicted)=0.992 · p(recorded)=0.007 · Other disease / proposed exclusion

CT · WL 40 / WW 400
Axial · model index 118

Predicted-class CAM

Recorded-class CAM

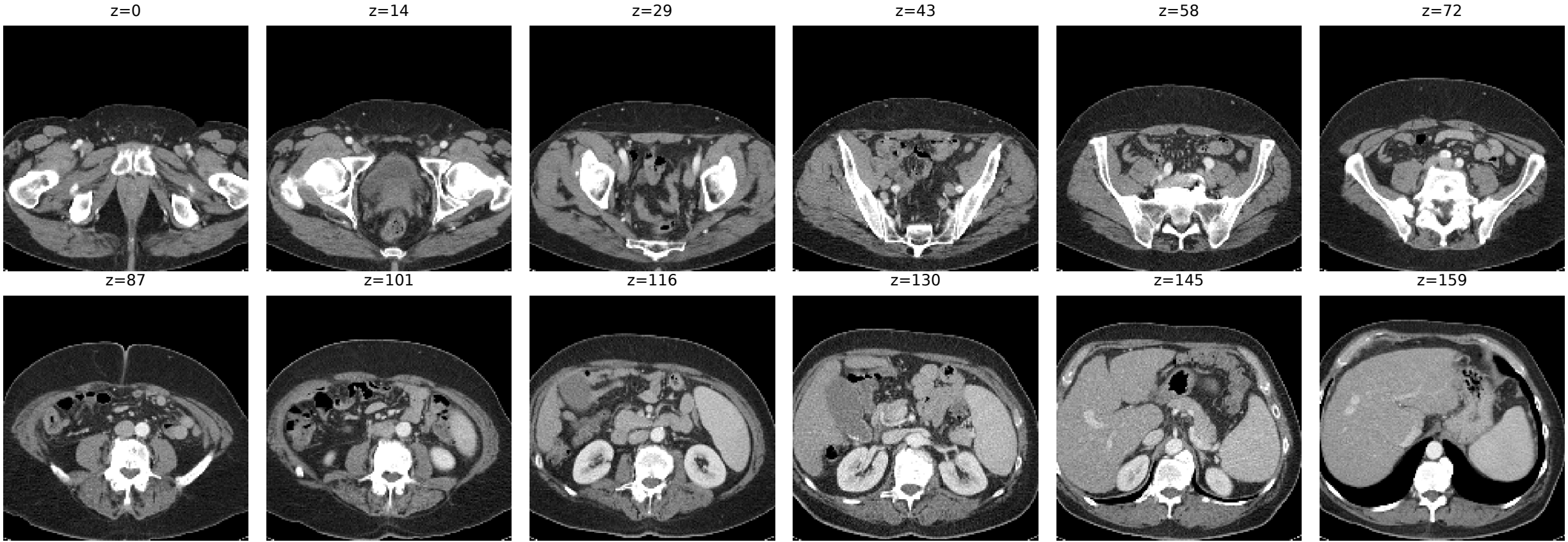
Signed CAM contrast



Coronal · model index 66

Sagittal · model index 26

LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R03: model-input axial overview | Normal → Cholezystitis
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R04 | Other disease / proposed exclusion

Recorded class: Normal | FlexiCT prediction: Appendicitis

Radiologist interpretation

Crohn disease exacerbation with surrounding inflammation and tumor infiltration.

Interpretation of the reread

Reviewer recommends exclusion

Prediction and explanation diagnostics

Predicted-class probability	0.9647
Recorded-class probability	0.0296
Fold votes for ensemble prediction	5
Top-mask probability drop	0.9630
Random-mask mean probability drop	0.5346
Positive drop exceeds random	True
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

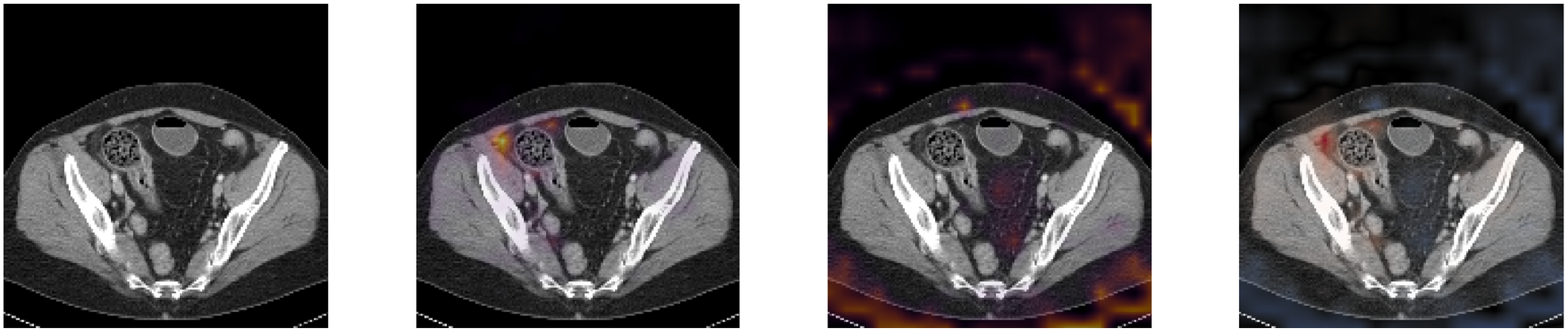
R04 · Recorded: Normal · Predicted: Appendicitis
p(predicted)=0.965 · p(recorded)=0.030 · Other disease / proposed exclusion

CT · WL 40 / WW 400
Axial · model index 34

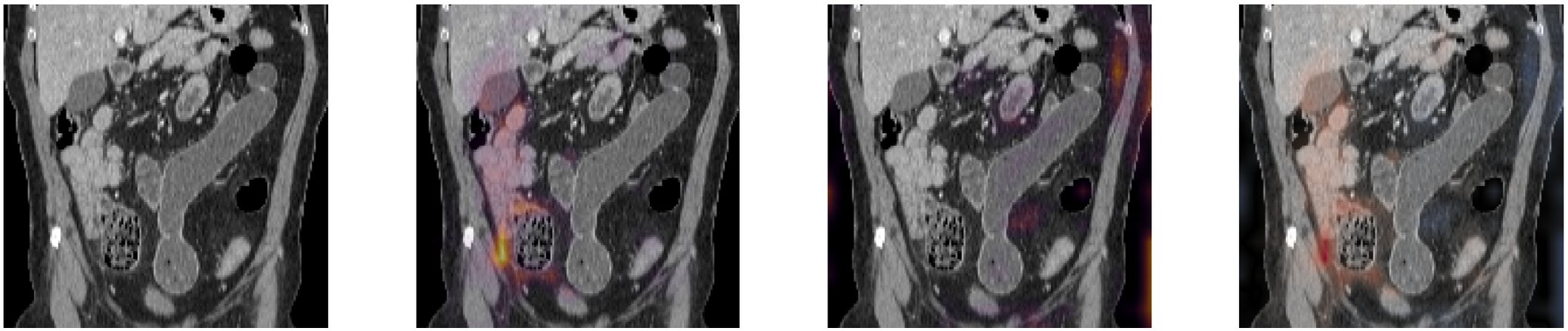
Predicted-class CAM

Recorded-class CAM

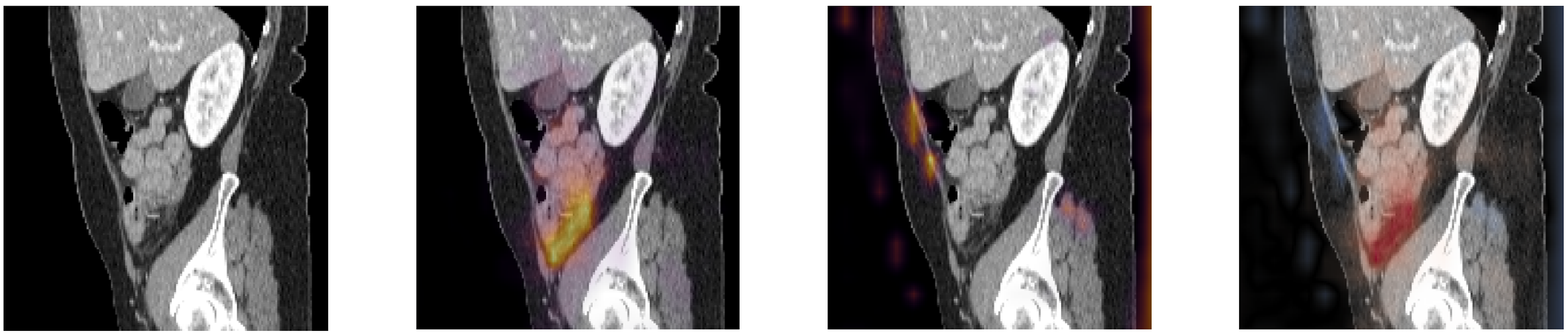
Signed CAM contrast



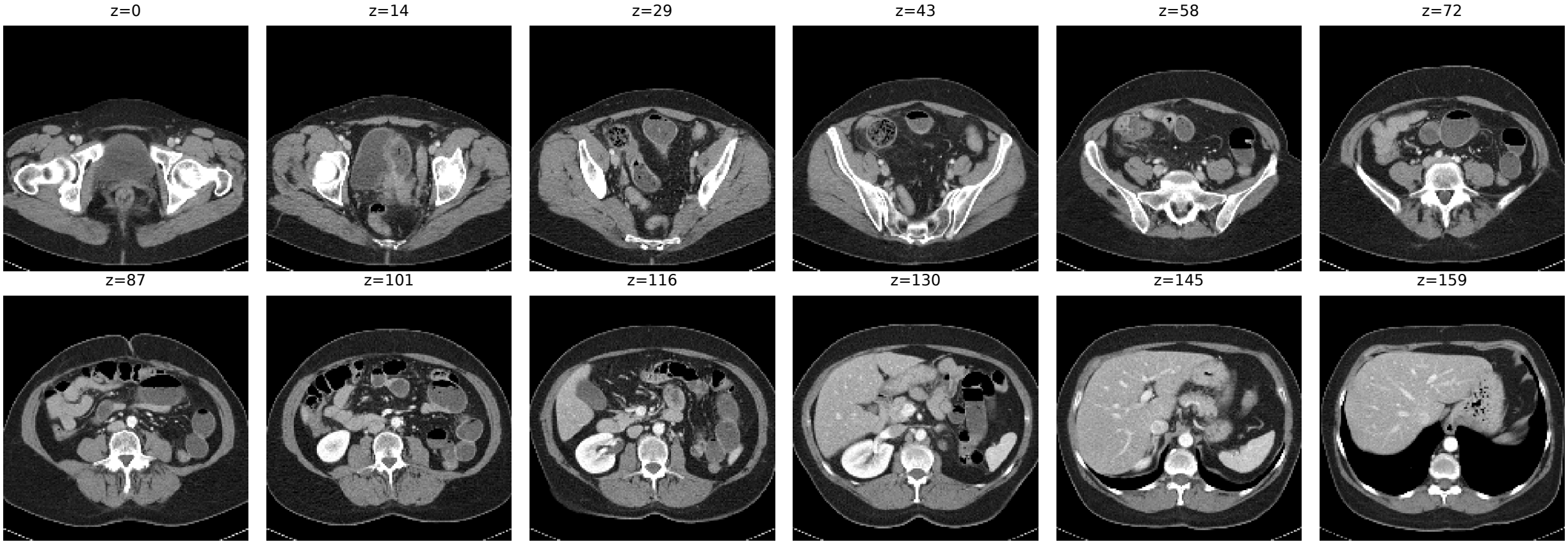
Coronal · model index 67



Sagittal · model index 42



LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R04: model-input axial overview | Normal → Appendicitis
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R05 | Target diagnosis not supported

Recorded class: Sigmoid | FlexiCT prediction: Normal

Radiologist interpretation

Incarcerated left inguinal hernia involving sigmoid; no clinical or operative proof of diverticulitis.

Interpretation of the reread

Absence of diverticulitis does not establish a normal abdomen

Prediction and explanation diagnostics

Predicted-class probability	0.9546
Recorded-class probability	0.0017
Fold votes for ensemble prediction	5
Top-mask probability drop	-0.0033
Random-mask mean probability drop	-0.0405
Positive drop exceeds random	False
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

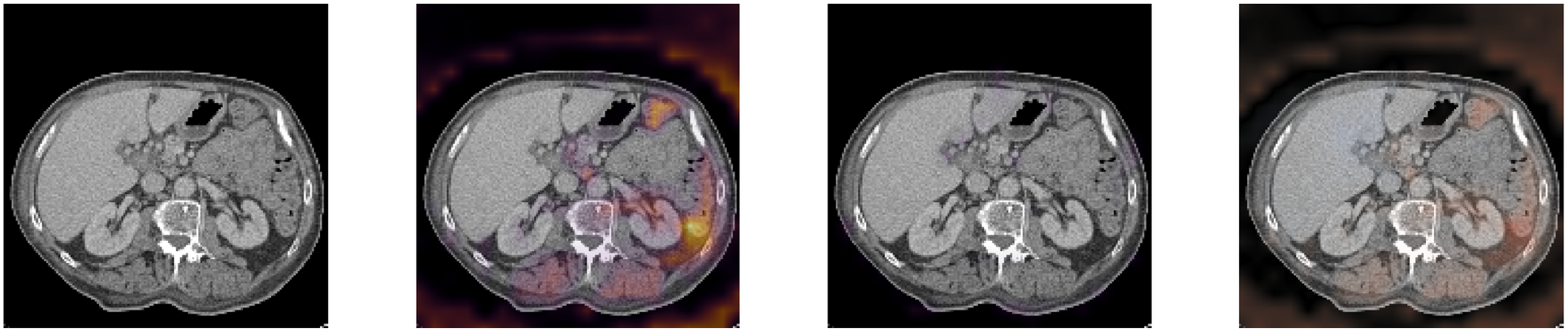
R05 · Recorded: Sigmoid · Predicted: Normal
p(predicted)=0.955 · p(recorded)=0.002 · Target diagnosis not supported

CT · WL 40 / WW 400
Axial · model index 125

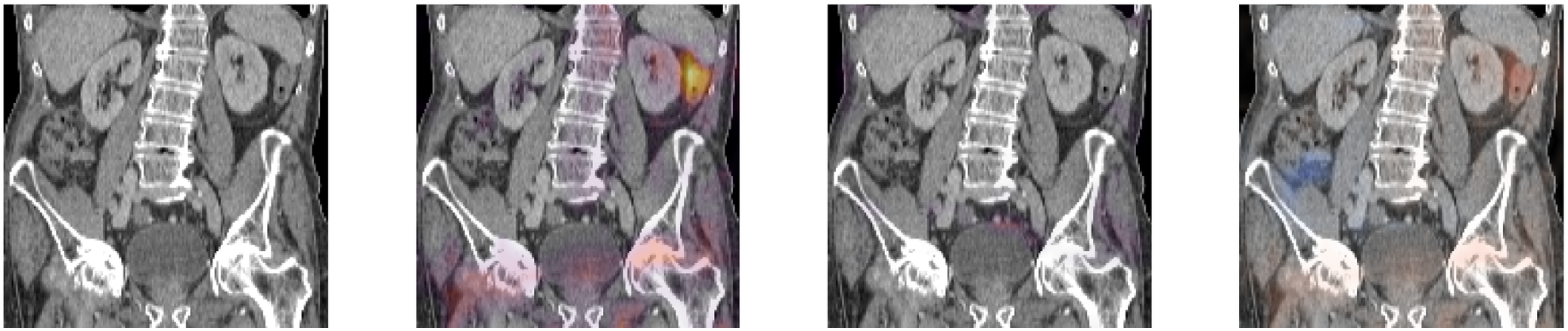
Predicted-class CAM

Recorded-class CAM

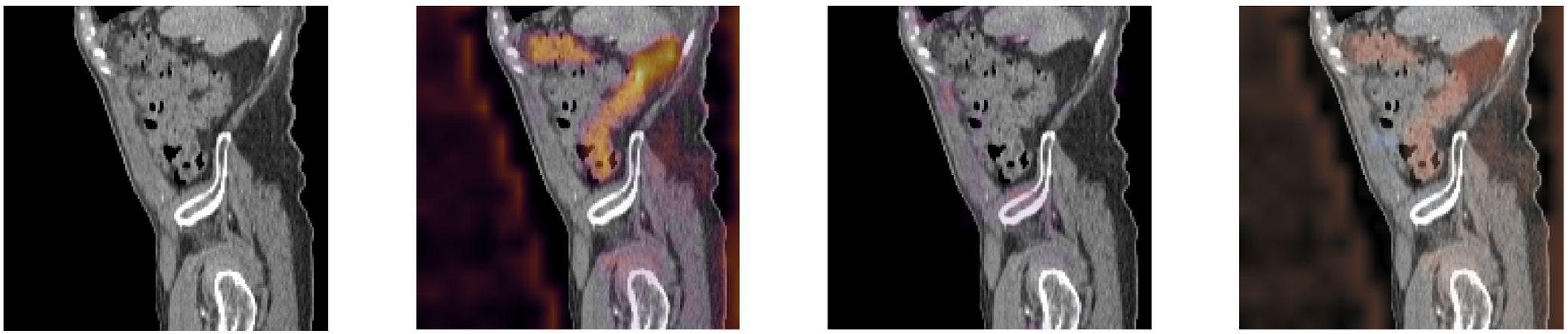
Signed CAM contrast



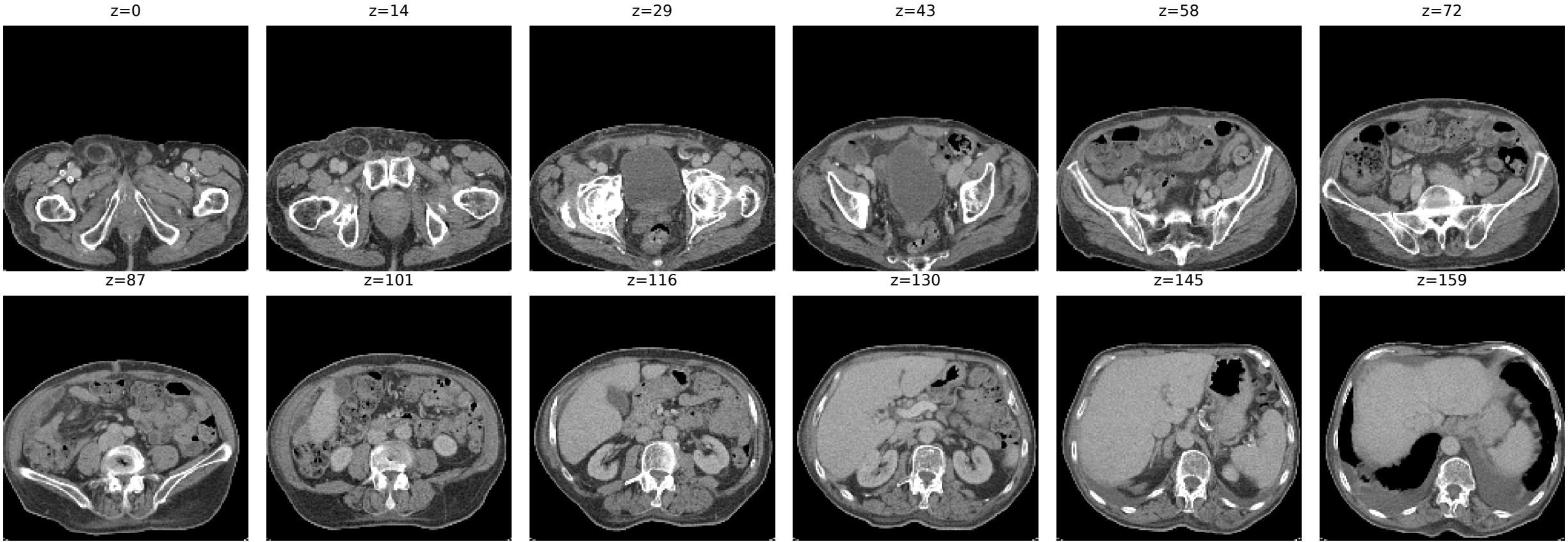
Coronal · model index 109



Sagittal · model index 134



LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R05: model-input axial overview | Sigmoid → Normal
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R06 | Other disease / proposed exclusion

Recorded class: Appendicitis | FlexiCT prediction: Normal

Radiologist interpretation

Malignancy-associated rectal obstruction, splenomegaly and ascites; no appendicitis.

Interpretation of the reread

Reviewer recommends exclusion

Prediction and explanation diagnostics

Predicted-class probability	0.9479
Recorded-class probability	0.0231
Fold votes for ensemble prediction	5
Top-mask probability drop	-0.0066
Random-mask mean probability drop	-0.0372
Positive drop exceeds random	False
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

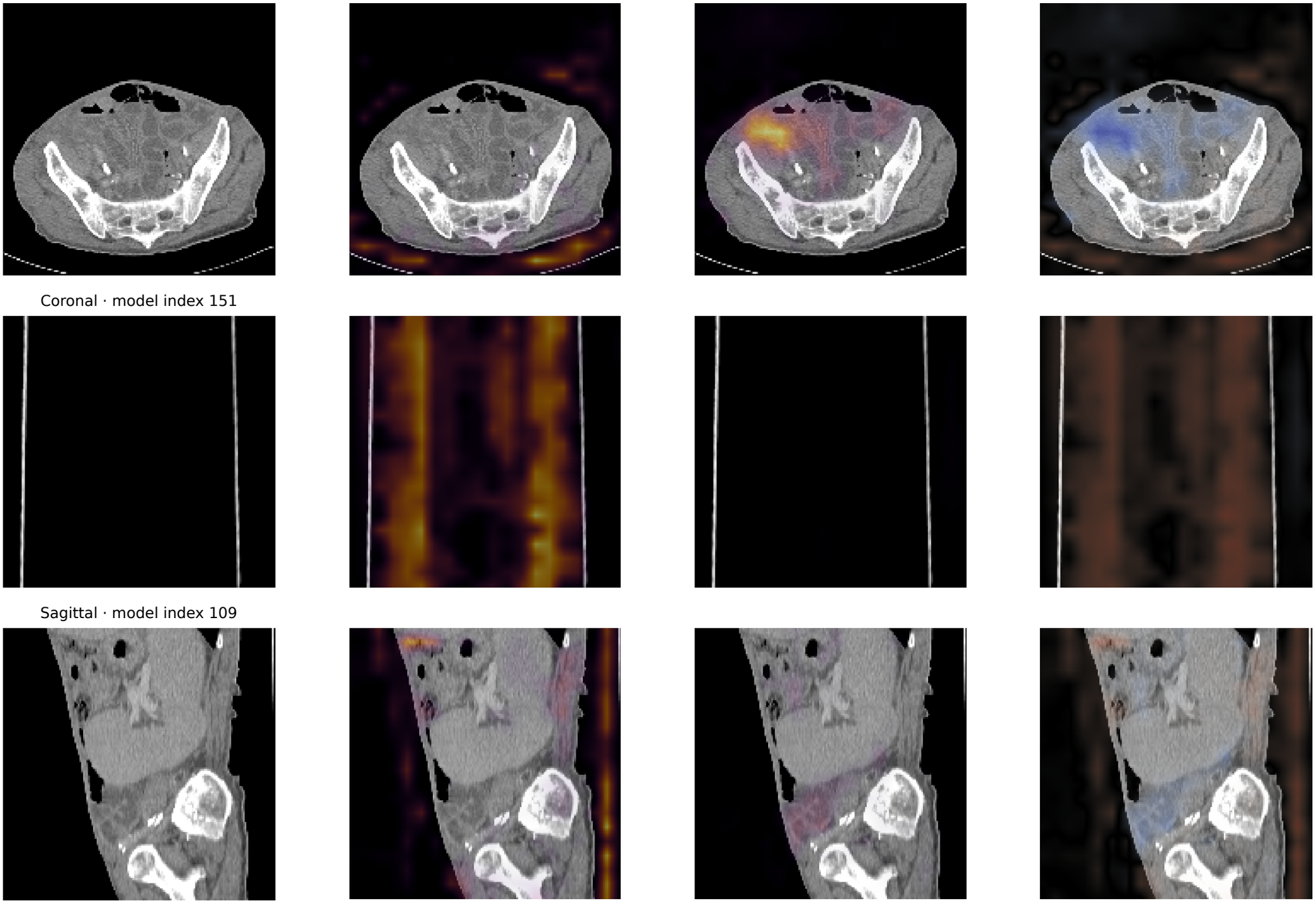
R06 · Recorded: Appendicitis · Predicted: Normal
p(predicted)=0.948 · p(recorded)=0.023 · Other disease / proposed exclusion

CT · WL 40 / WW 400
Axial · model index 42

Predicted-class CAM

Recorded-class CAM

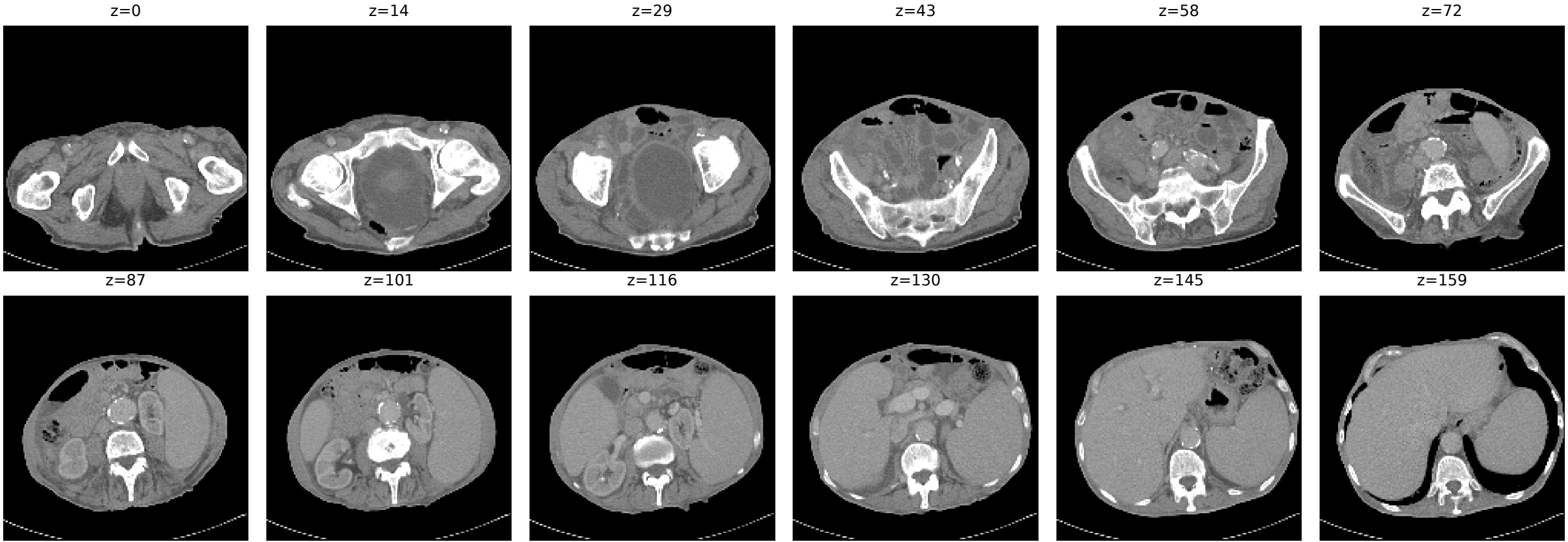
Signed CAM contrast



Coronal · model index 151

Sagittal · model index 109

LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R06: model-input axial overview | Appendicitis → Normal
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R07 | Likely error / uncertain reference

Recorded class: Cholezystitis | FlexiCT prediction: Normal

Radiologist interpretation

Cholecystitis likely but not proven; nearby postoperative stoma may mimic inflammation.

Interpretation of the reread

Reviewer retains case; model likely wrong

Prediction and explanation diagnostics

Predicted-class probability	0.9188
Recorded-class probability	0.0020
Fold votes for ensemble prediction	5
Top-mask probability drop	-0.0248
Random-mask mean probability drop	-0.0374
Positive drop exceeds random	False
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

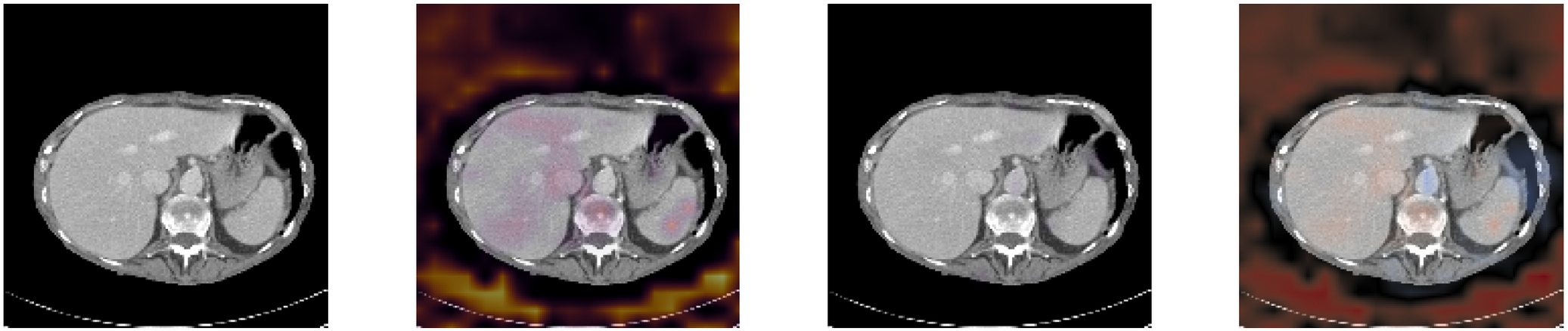
R07 · Recorded: Cholezystitis · Predicted: Normal
p(predicted)=0.919 · p(recorded)=0.002 · Likely error / uncertain reference

CT · WL 40 / WW 400
Axial · model index 151

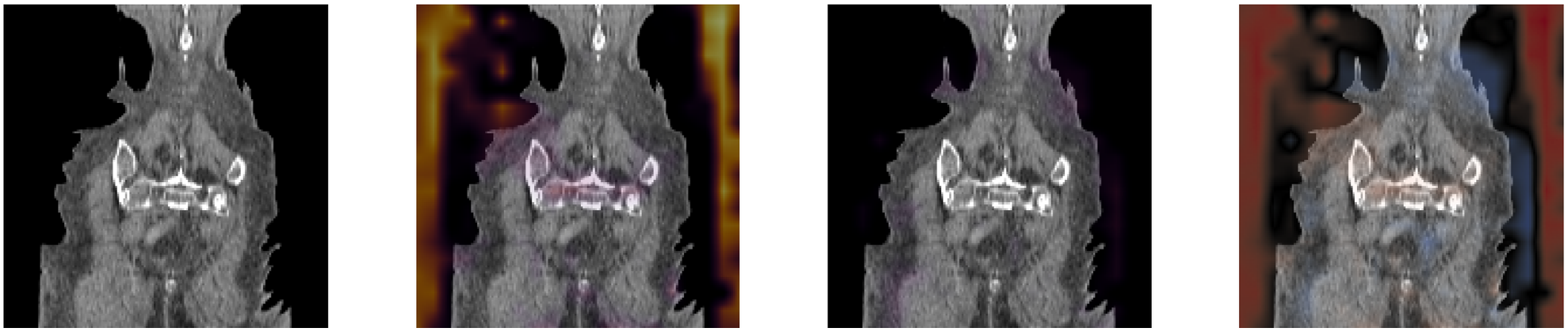
Predicted-class CAM

Recorded-class CAM

Signed CAM contrast

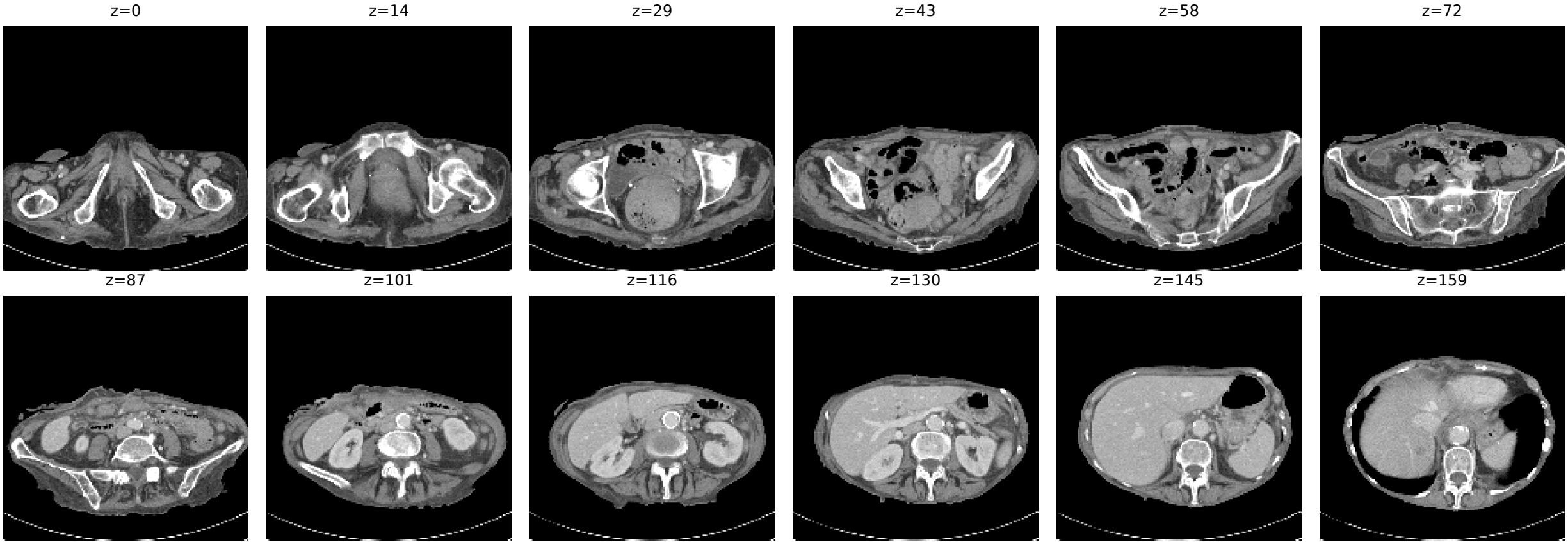


Coronal · model index 134



Sagittal · model index 150

LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R07: model-input axial overview | Cholezystitis → Normal
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R08 | Overlapping target diagnoses

Recorded class: Sigmoid | FlexiCT prediction: Perforation

Radiologist interpretation

Acute and chronic sigmoid diverticulitis with perforation.

Interpretation of the reread

Single-label task conflates coexisting diagnoses

Prediction and explanation diagnostics

Predicted-class probability	0.8934
Recorded-class probability	0.1028
Fold votes for ensemble prediction	5
Top-mask probability drop	-0.0185
Random-mask mean probability drop	-0.0423
Positive drop exceeds random	False
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

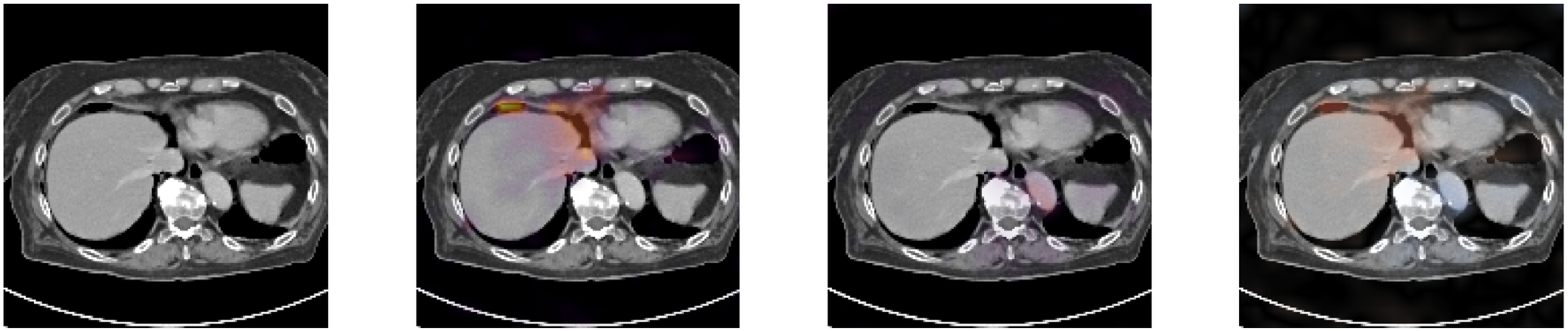
R08 · Recorded: Sigmoid · Predicted: Perforation
p(predicted)=0.893 · p(recorded)=0.103 · Overlapping target diagnoses

CT · WL 40 / WW 400
Axial · model index 151

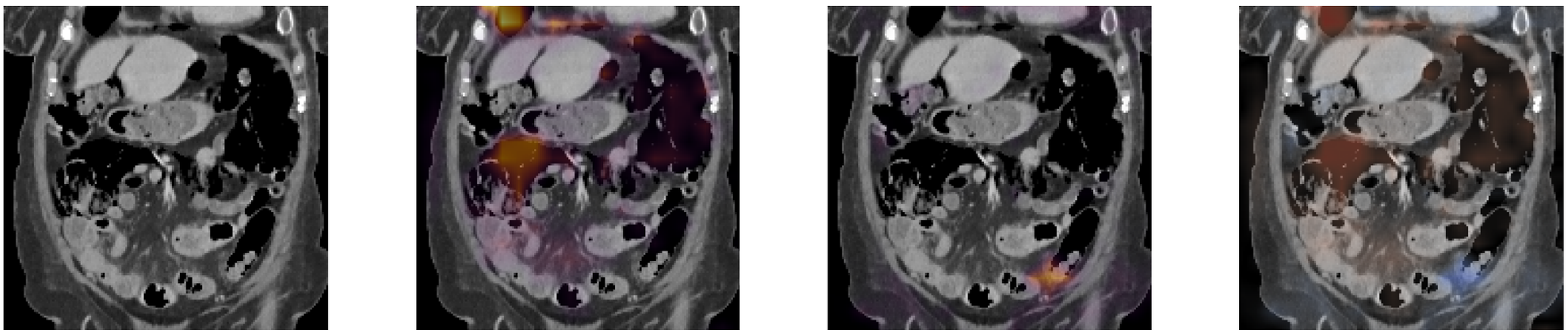
Predicted-class CAM

Recorded-class CAM

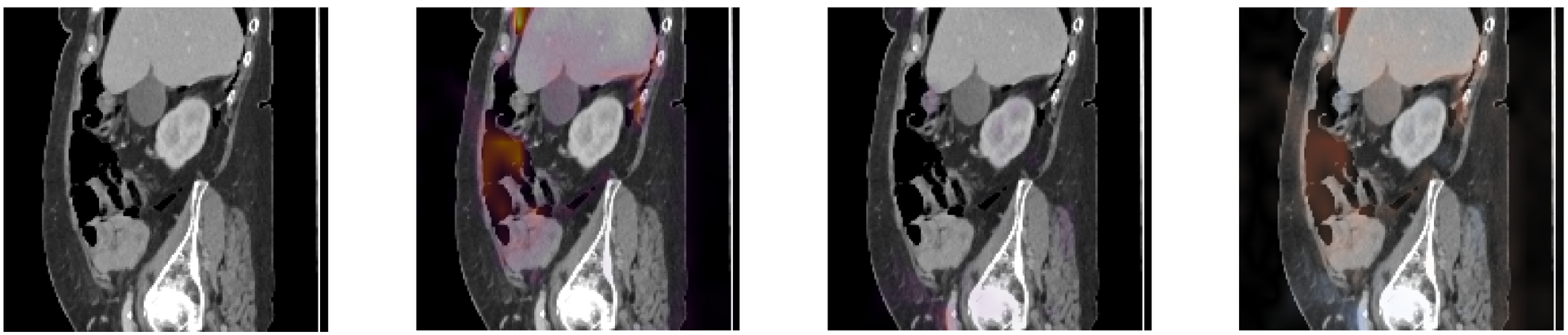
Signed CAM contrast



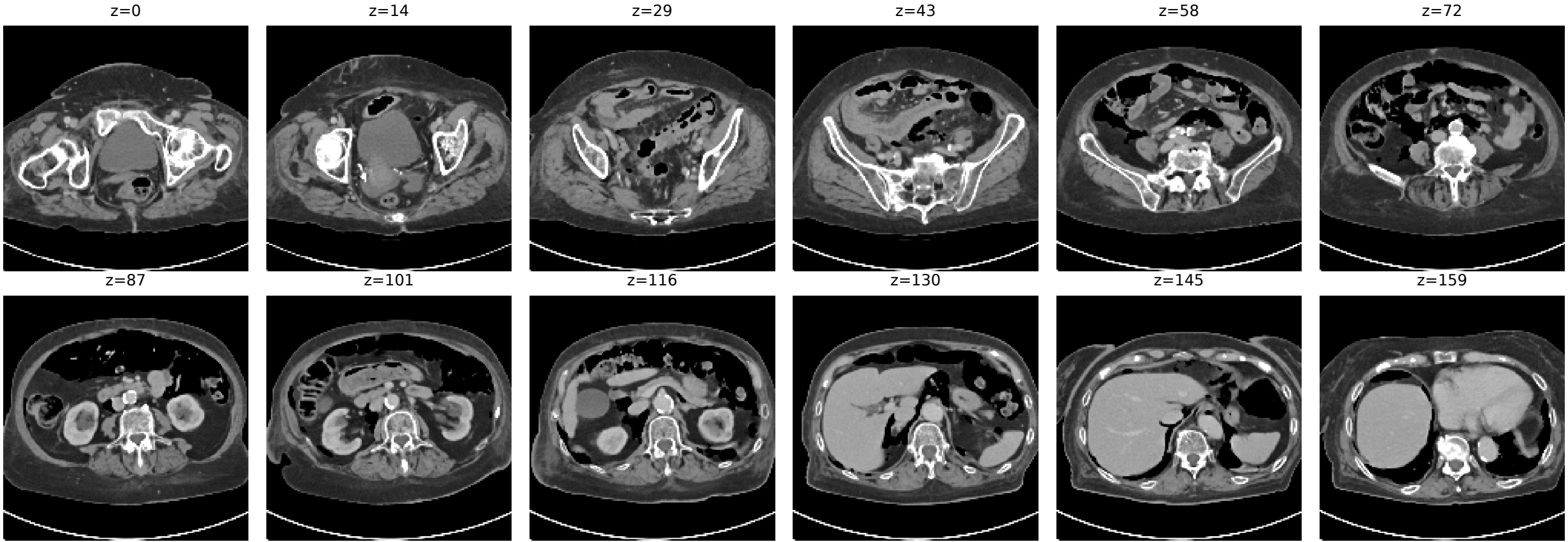
Coronal · model index 50



Sagittal · model index 42



LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R08: model-input axial overview | Sigmoid → Perforation
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R09 | Confirmed model error

Recorded class: Cholezystitis | FlexiCT prediction: Normal

Radiologist interpretation

Subtle cholecystitis clinically proven, with cholestasis.

Interpretation of the reread

Recorded disease retained

Prediction and explanation diagnostics

Predicted-class probability	0.8738
Recorded-class probability	0.0948
Fold votes for ensemble prediction	5
Top-mask probability drop	-0.0974
Random-mask mean probability drop	-0.1101
Positive drop exceeds random	False
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

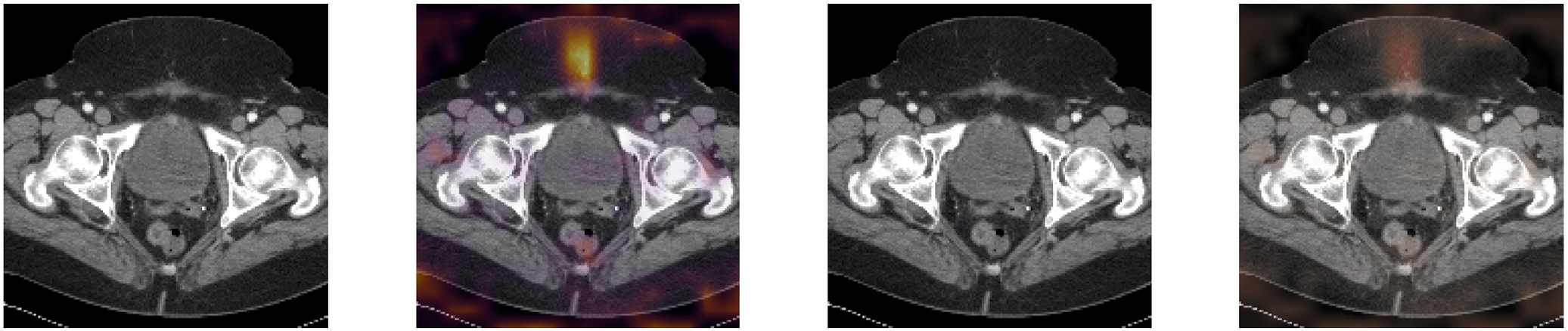
R09 · Recorded: Cholezystitis · Predicted: Normal
p(predicted)=0.874 · p(recorded)=0.095 · Confirmed model error

CT · WL 40 / WW 400
Axial · model index 9

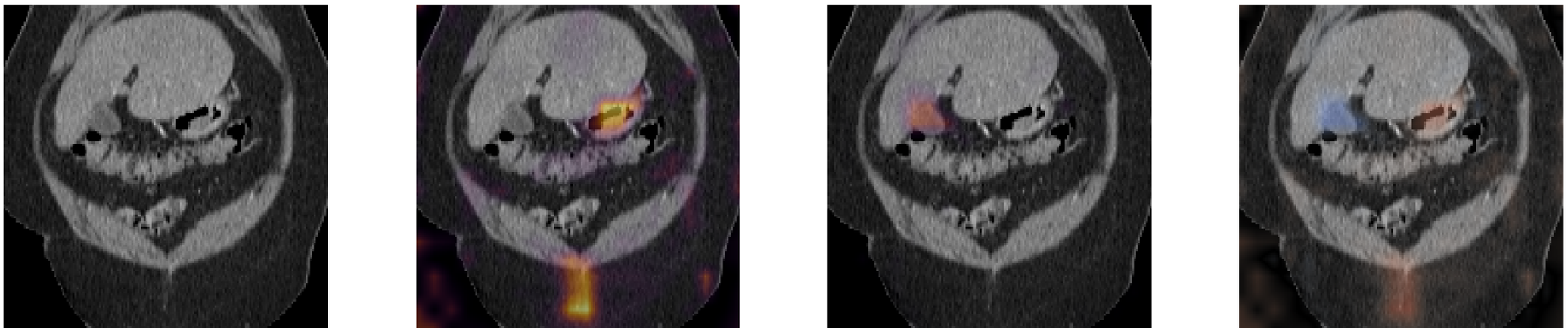
Predicted-class CAM

Recorded-class CAM

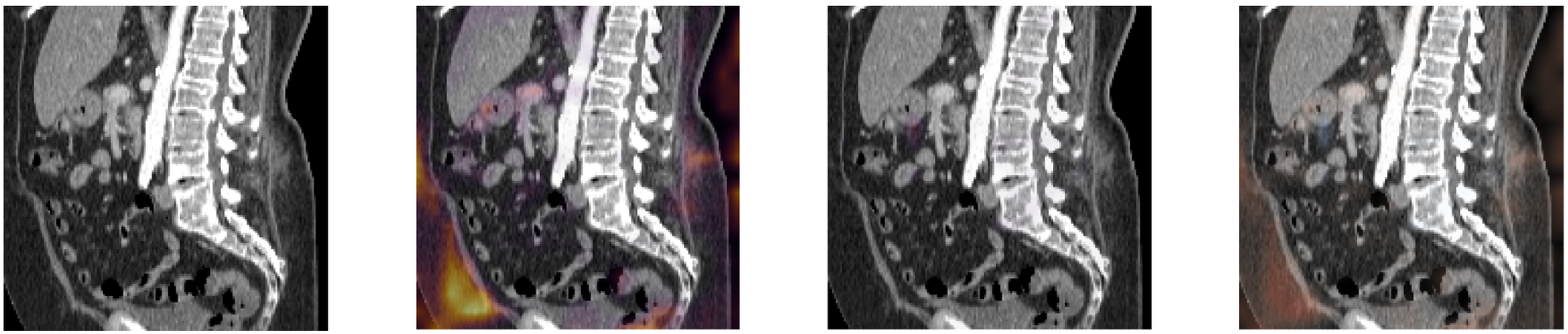
Signed CAM contrast



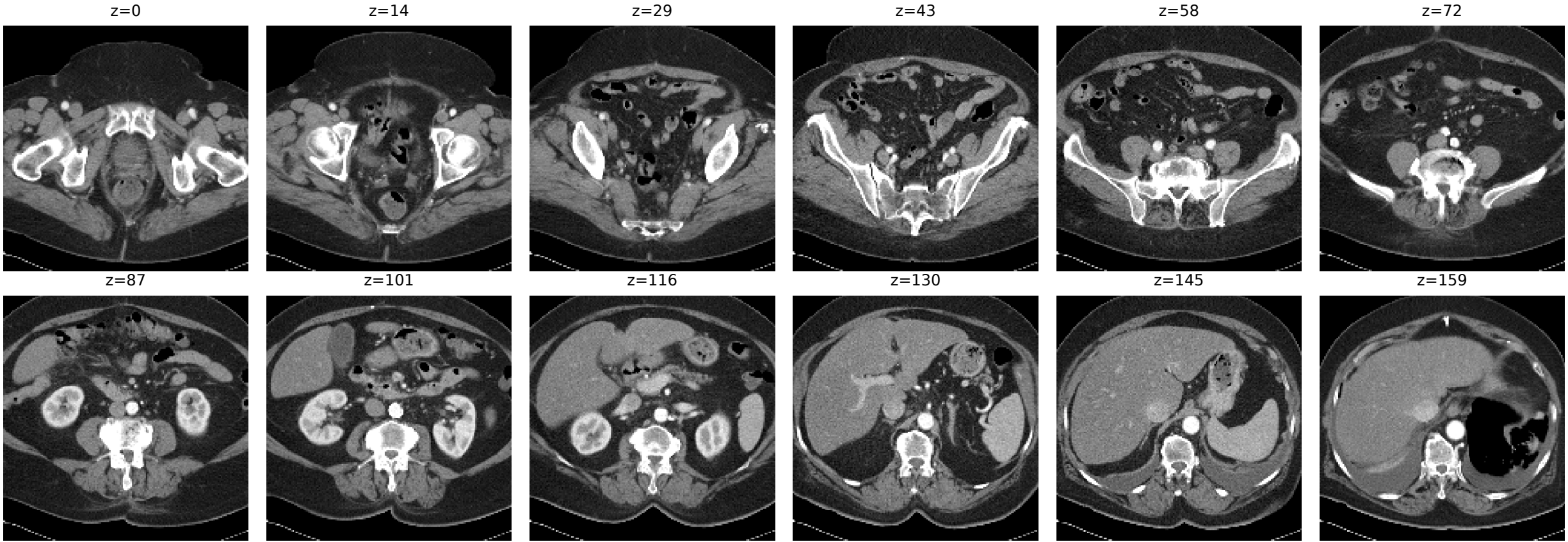
Coronal · model index 25



Sagittal · model index 83



LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R09: model-input axial overview | Cholezystitis → Normal
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R10 | Confirmed model error

Recorded class: Cholezystitis | FlexiCT prediction: Normal

Radiologist interpretation

Proven cholecystitis with complex anatomy, displaced kidney, hernias, sarcopenia and chronic inflammation.

Interpretation of the reread

Recorded disease retained

Prediction and explanation diagnostics

Predicted-class probability	0.8702
Recorded-class probability	0.0661
Fold votes for ensemble prediction	5
Top-mask probability drop	0.1225
Random-mask mean probability drop	0.0215
Positive drop exceeds random	True
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

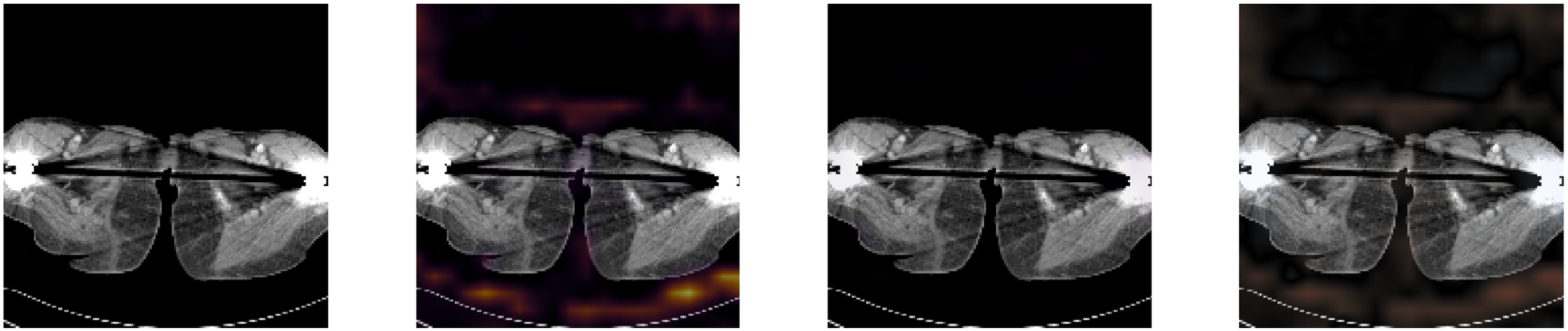
R10 · Recorded: Cholezystitis · Predicted: Normal
p(predicted)=0.870 · p(recorded)=0.066 · Confirmed model error

CT · WL 40 / WW 400
Axial · model index 0

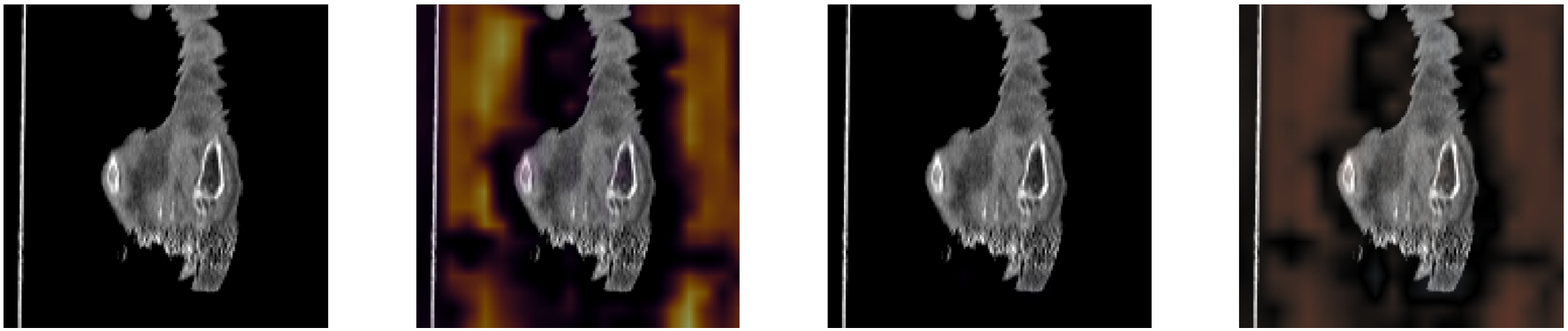
Predicted-class CAM

Recorded-class CAM

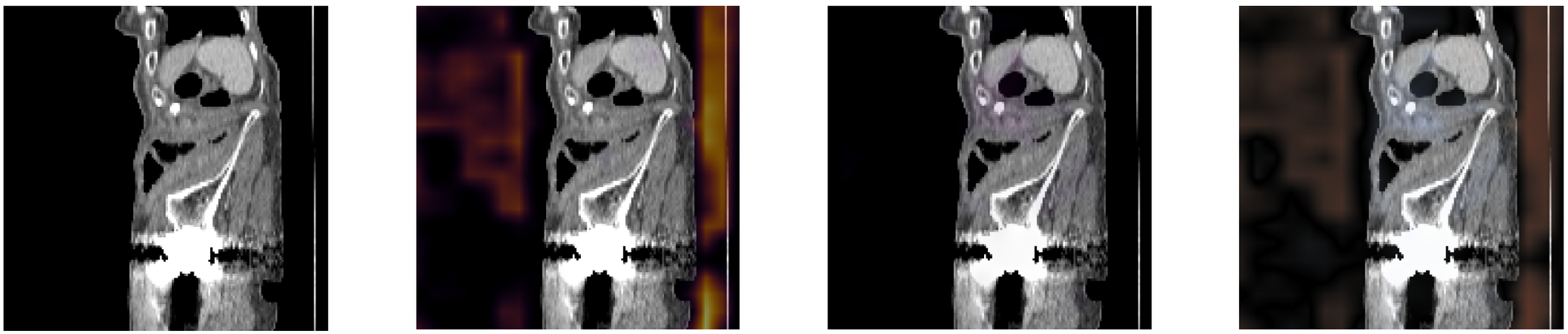
Signed CAM contrast



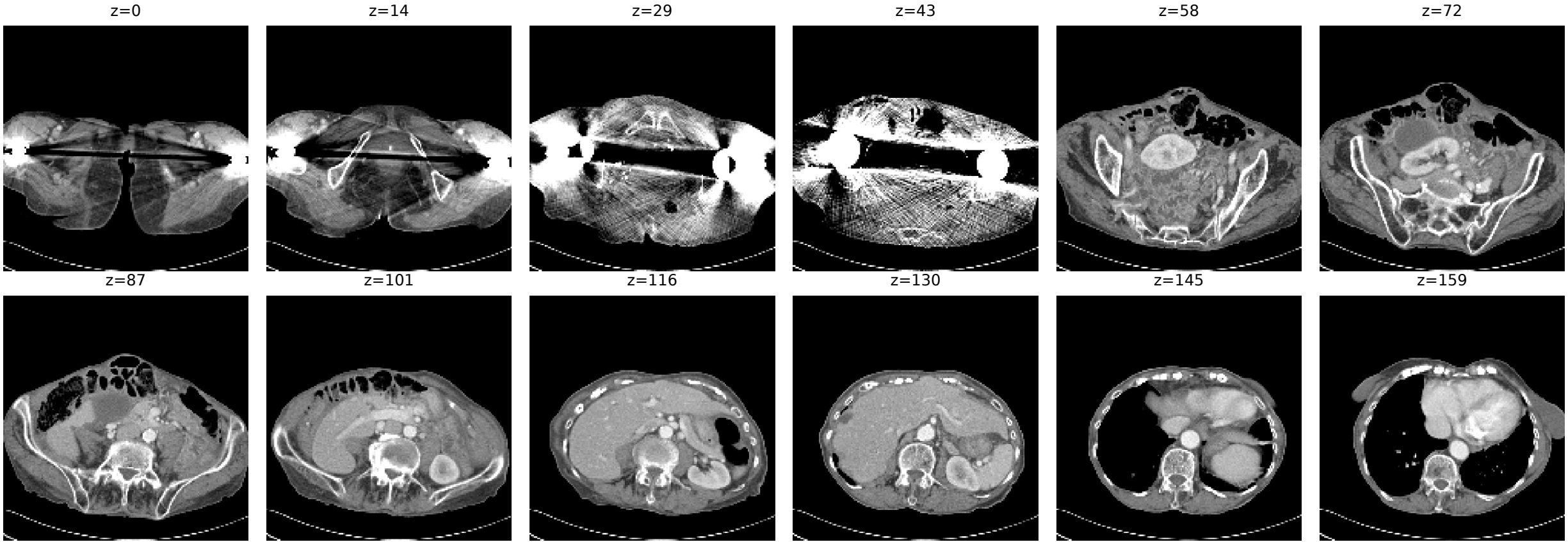
Coronal · model index 142



Sagittal · model index 134



LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R10: model-input axial overview | Cholezystitis → Normal
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R11 | Confirmed model error

Recorded class: Normal | FlexiCT prediction: Appendicitis

Radiologist interpretation

No pathology on either review.

Interpretation of the reread

Recorded Normal label retained

Prediction and explanation diagnostics

Predicted-class probability	0.8670
Recorded-class probability	0.1269
Fold votes for ensemble prediction	5
Top-mask probability drop	0.8666
Random-mask mean probability drop	0.8664
Positive drop exceeds random	True
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

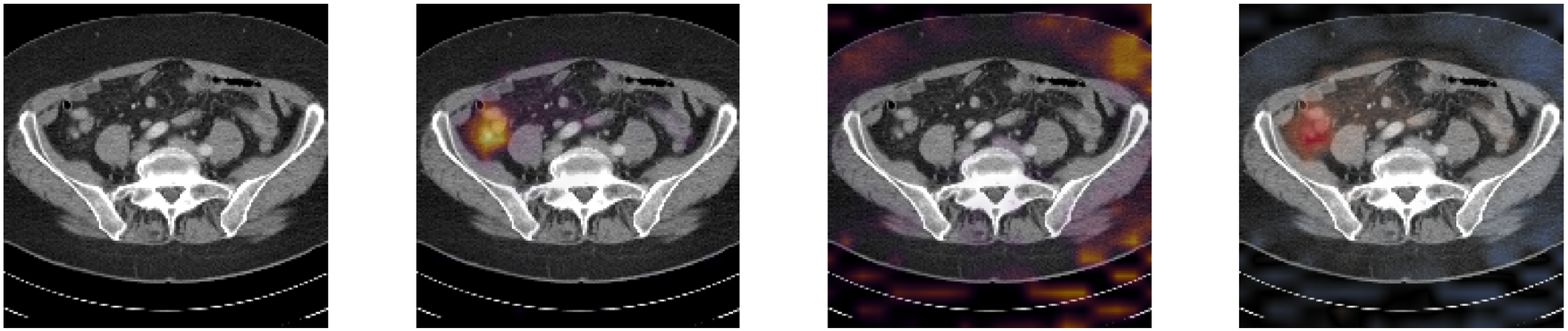
R11 · Recorded: Normal · Predicted: Appendicitis
p(predicted)=0.867 · p(recorded)=0.127 · Confirmed model error

CT · WL 40 / WW 400
Axial · model index 59

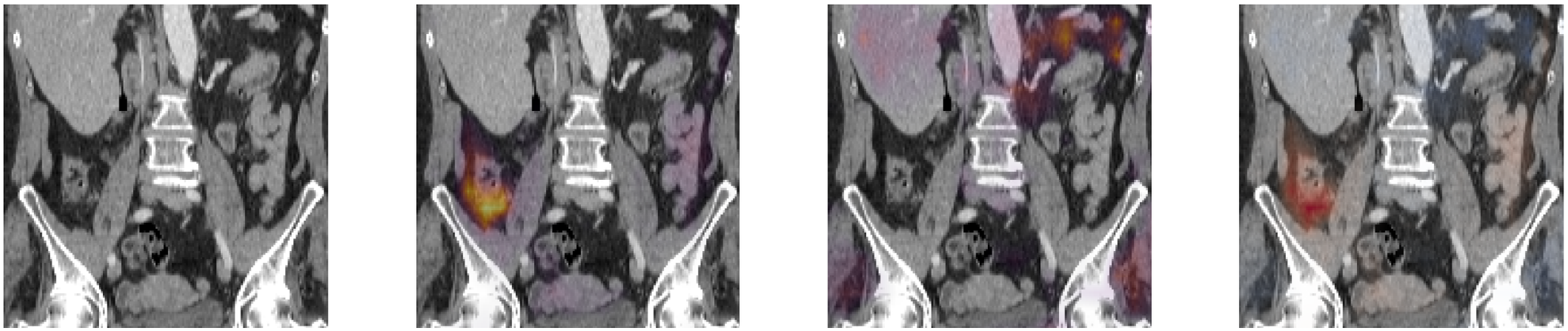
Predicted-class CAM

Recorded-class CAM

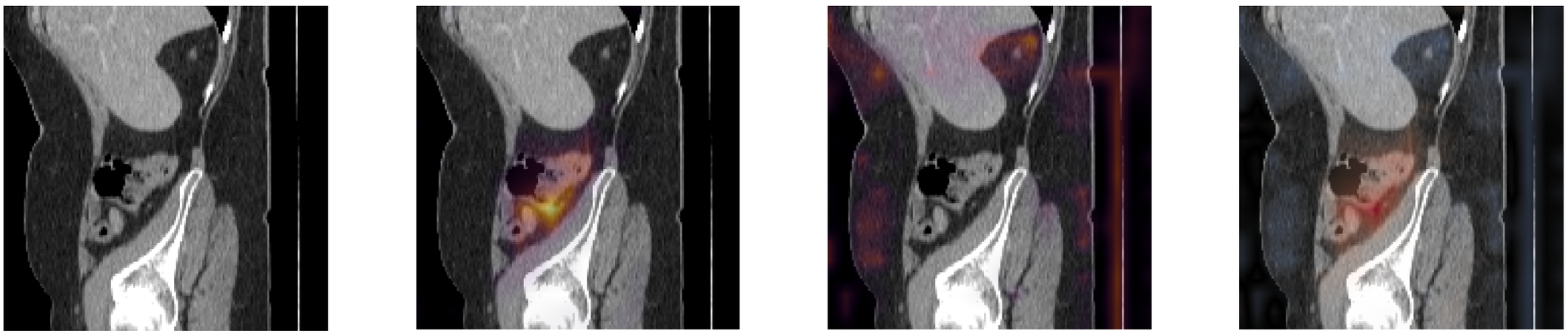
Signed CAM contrast



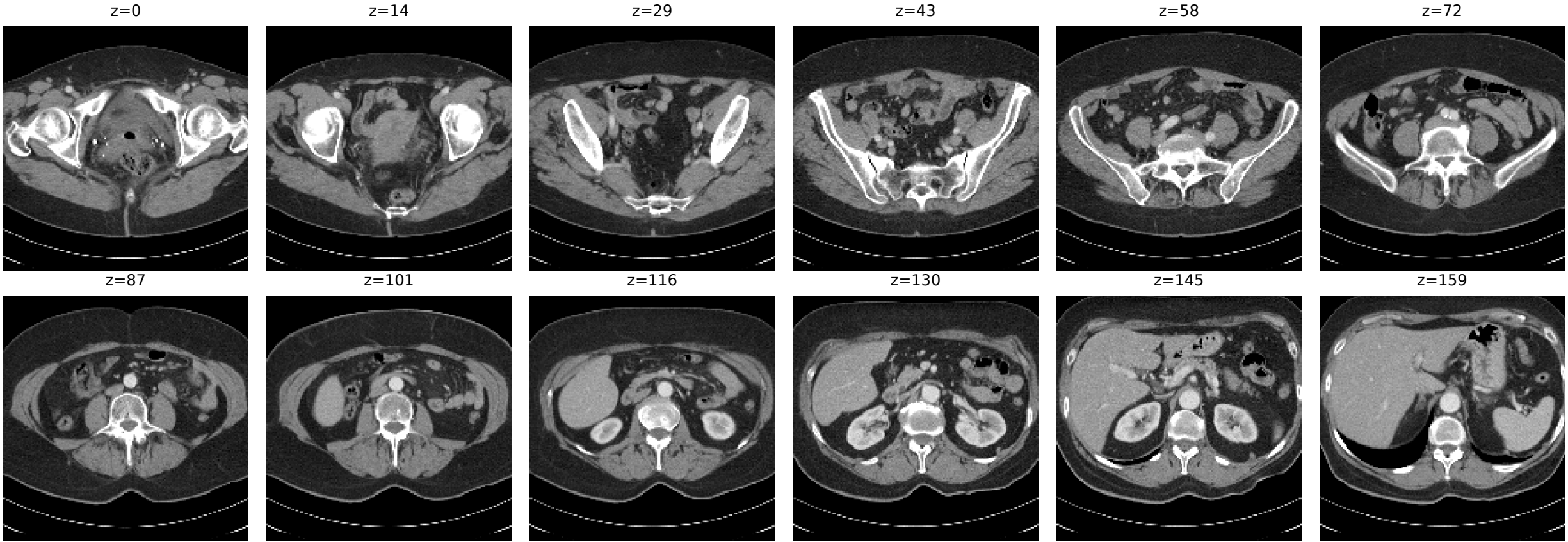
Coronal · model index 67



Sagittal · model index 34



LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R11: model-input axial overview | Normal → Appendicitis
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R12 | Target diagnosis not supported

Recorded class: Cholezystitis | FlexiCT prediction: Normal

Radiologist interpretation

Urosepsis and gallbladder hydrops; initial noncontrast CT diagnosis of cholecystitis was not supported.

Interpretation of the reread

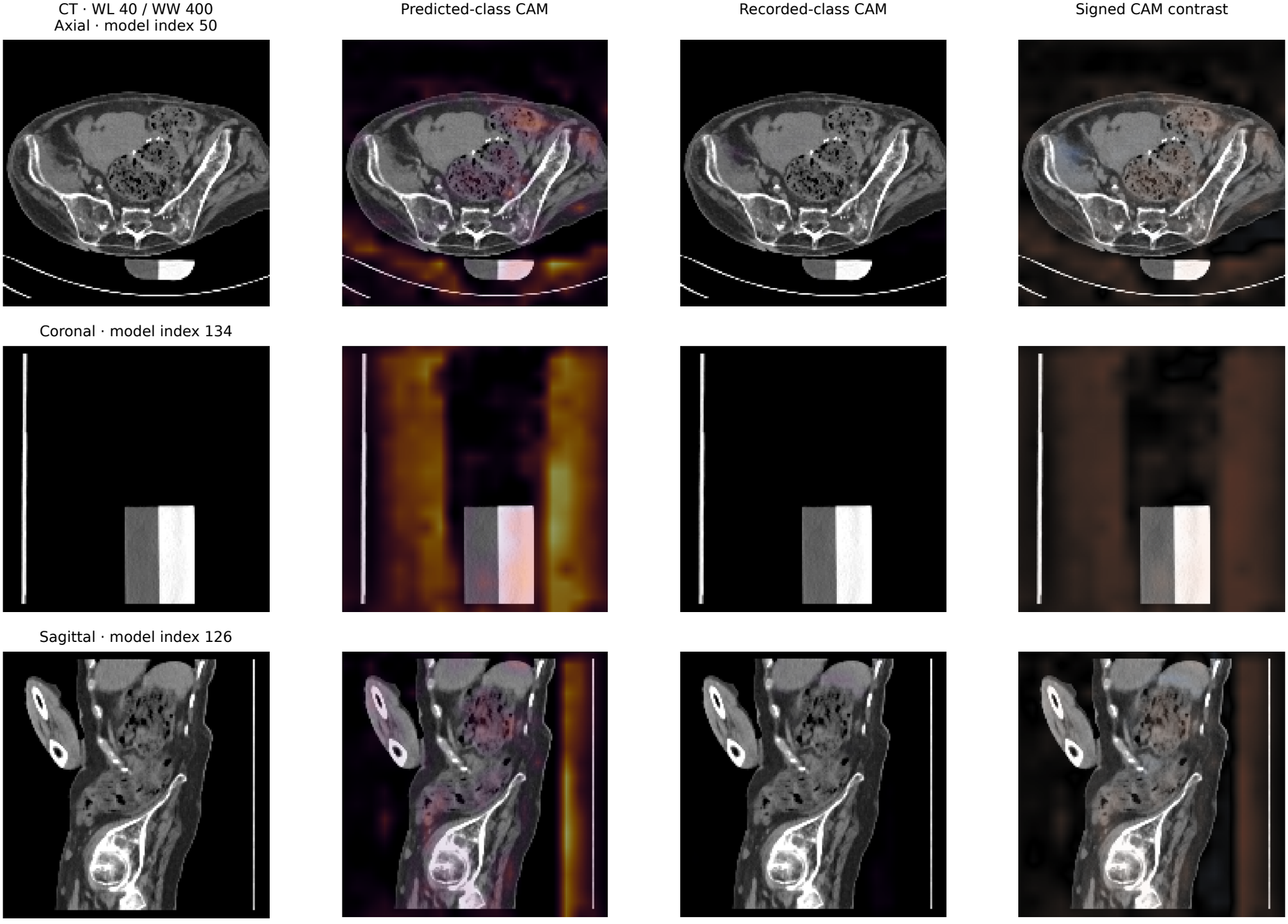
Absence of cholecystitis does not establish a normal abdomen

Prediction and explanation diagnostics

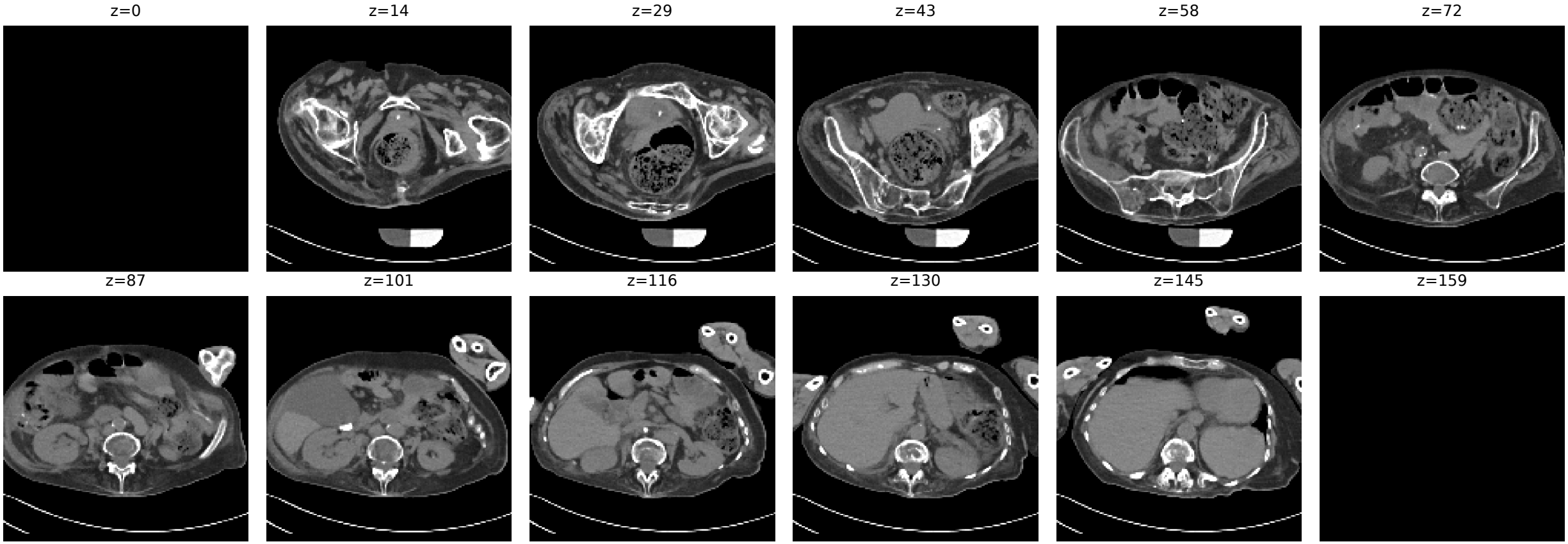
Predicted-class probability	0.8334
Recorded-class probability	0.0461
Fold votes for ensemble prediction	5
Top-mask probability drop	0.1511
Random-mask mean probability drop	0.0506
Positive drop exceeds random	True
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

R12 · Recorded: Cholezystitis · Predicted: Normal
p(predicted)=0.833 · p(recorded)=0.046 · Target diagnosis not supported



LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R12: model-input axial overview | Cholezystitis → Normal
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R13 | Confirmed model error

Recorded class: Normal | FlexiCT prediction: Sigmoid

Radiologist interpretation

No pathology on two reviews; a dense sigmoid clip is a proposed mimic.

Interpretation of the reread

Clip explanation is a reviewer hypothesis

Prediction and explanation diagnostics

Predicted-class probability	0.7918
Recorded-class probability	0.2050
Fold votes for ensemble prediction	5
Top-mask probability drop	0.7699
Random-mask mean probability drop	0.6524
Positive drop exceeds random	True
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

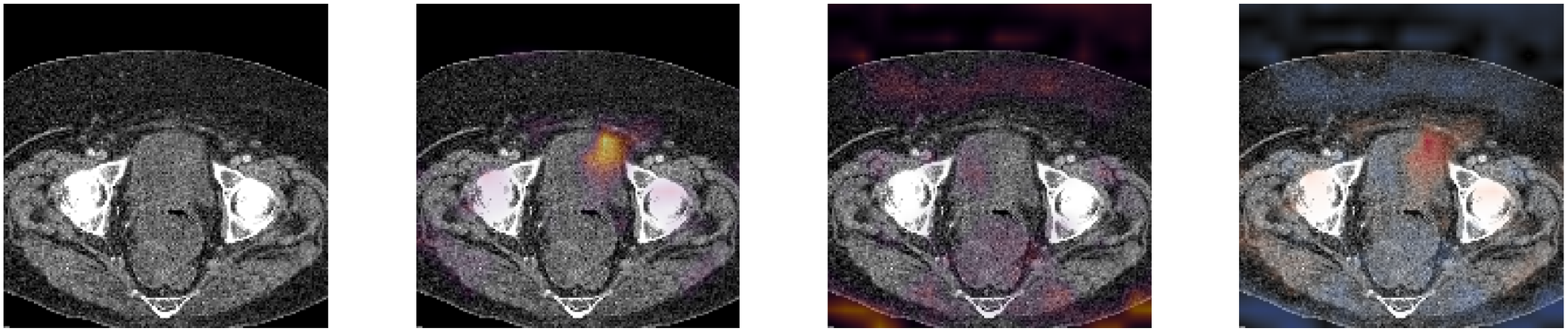
R13 · Recorded: Normal · Predicted: Sigmoid
p(predicted)=0.792 · p(recorded)=0.205 · Confirmed model error

CT · WL 40 / WW 400
Axial · model index 9

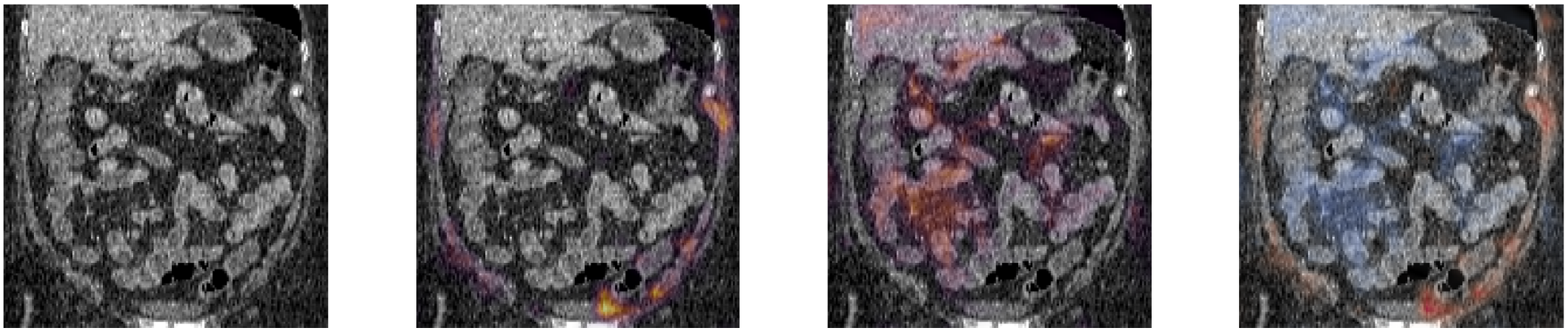
Predicted-class CAM

Recorded-class CAM

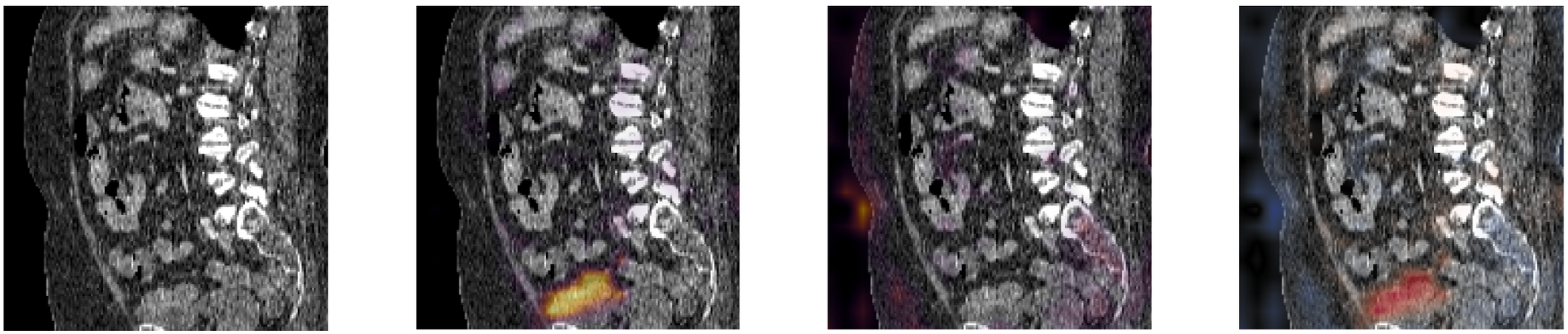
Signed CAM contrast



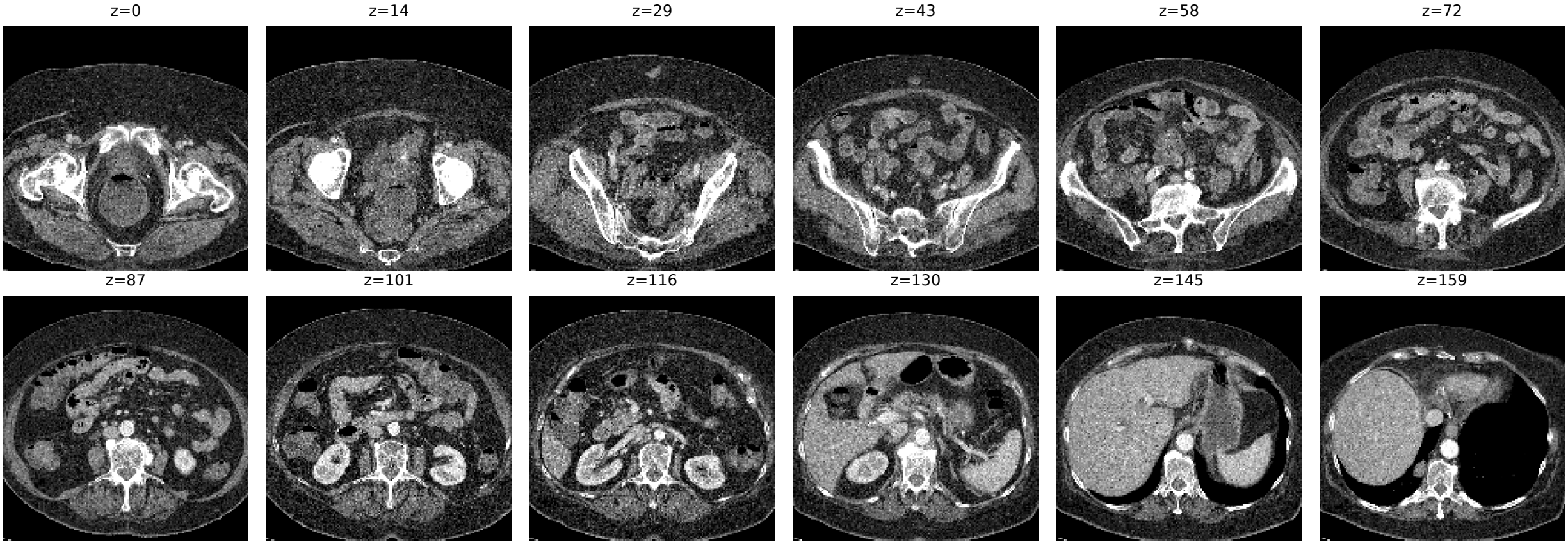
Coronal · model index 67



Sagittal · model index 92



LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R13: model-input axial overview | Normal → Sigmoid
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R14 | Confirmed model error

Recorded class: Perforation | FlexiCT prediction: Normal

Radiologist interpretation

Operatively confirmed covered duodenal perforation with large air/fluid collection and atypical anatomy.

Interpretation of the reread

Normal prediction is a true miss on reread

Prediction and explanation diagnostics

Predicted-class probability	0.7697
Recorded-class probability	0.0148
Fold votes for ensemble prediction	5
Top-mask probability drop	0.0531
Random-mask mean probability drop	0.0122
Positive drop exceeds random	True
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

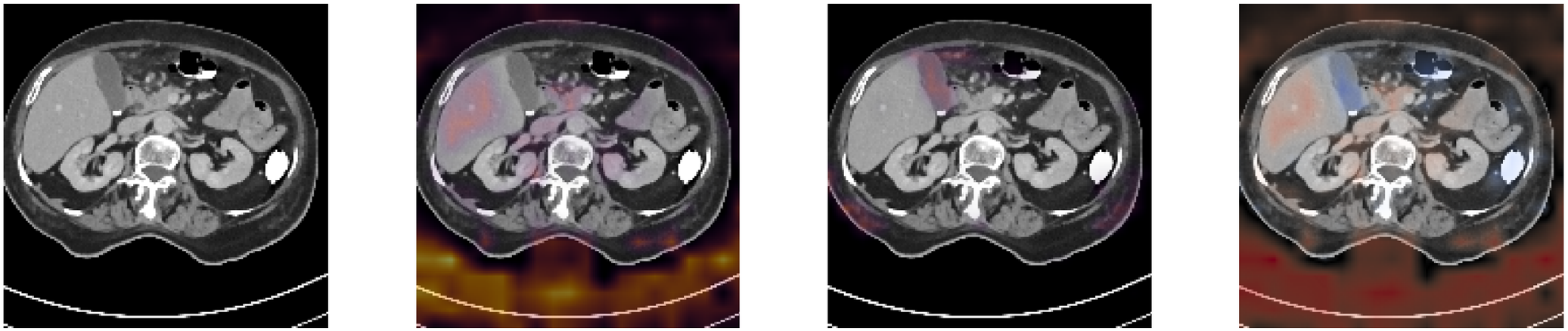
R14 · Recorded: Perforation · Predicted: Normal
p(predicted)=0.770 · p(recorded)=0.015 · Confirmed model error

CT · WL 40 / WW 400
Axial · model index 126

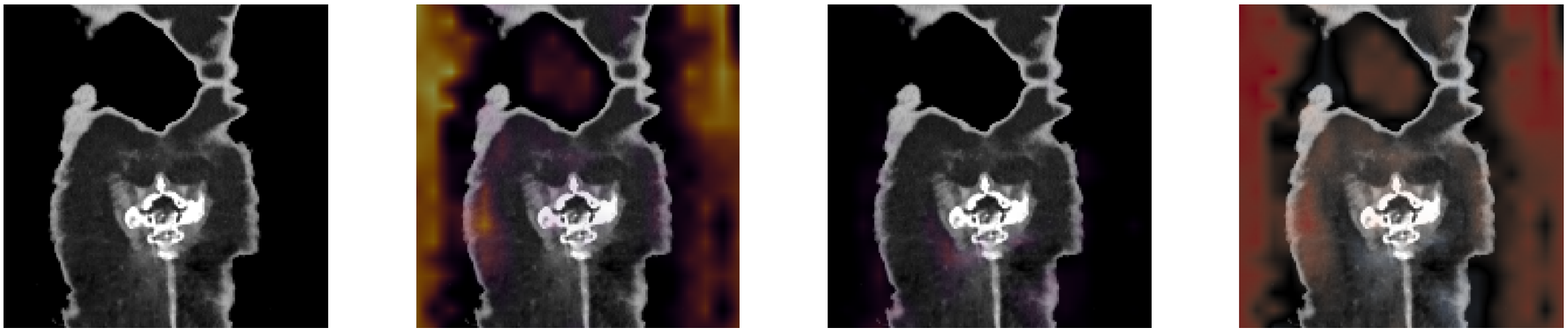
Predicted-class CAM

Recorded-class CAM

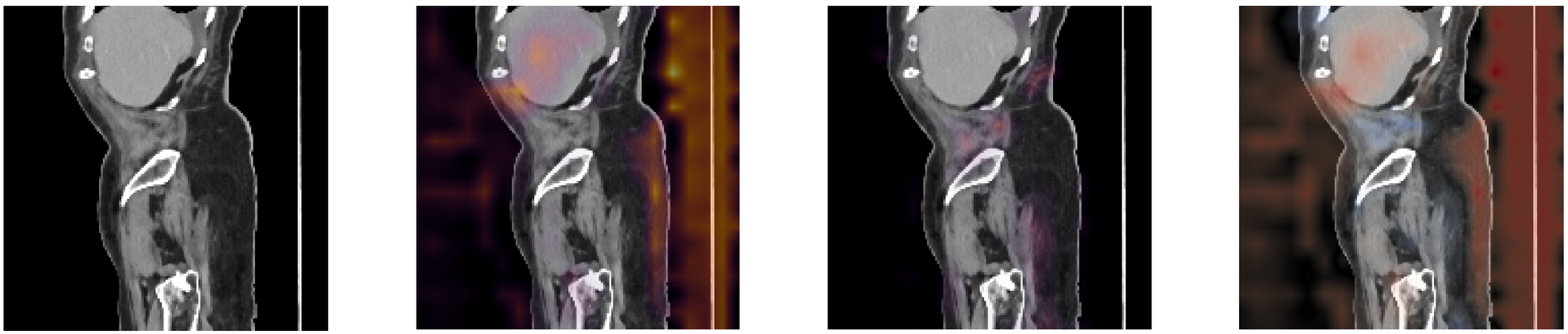
Signed CAM contrast



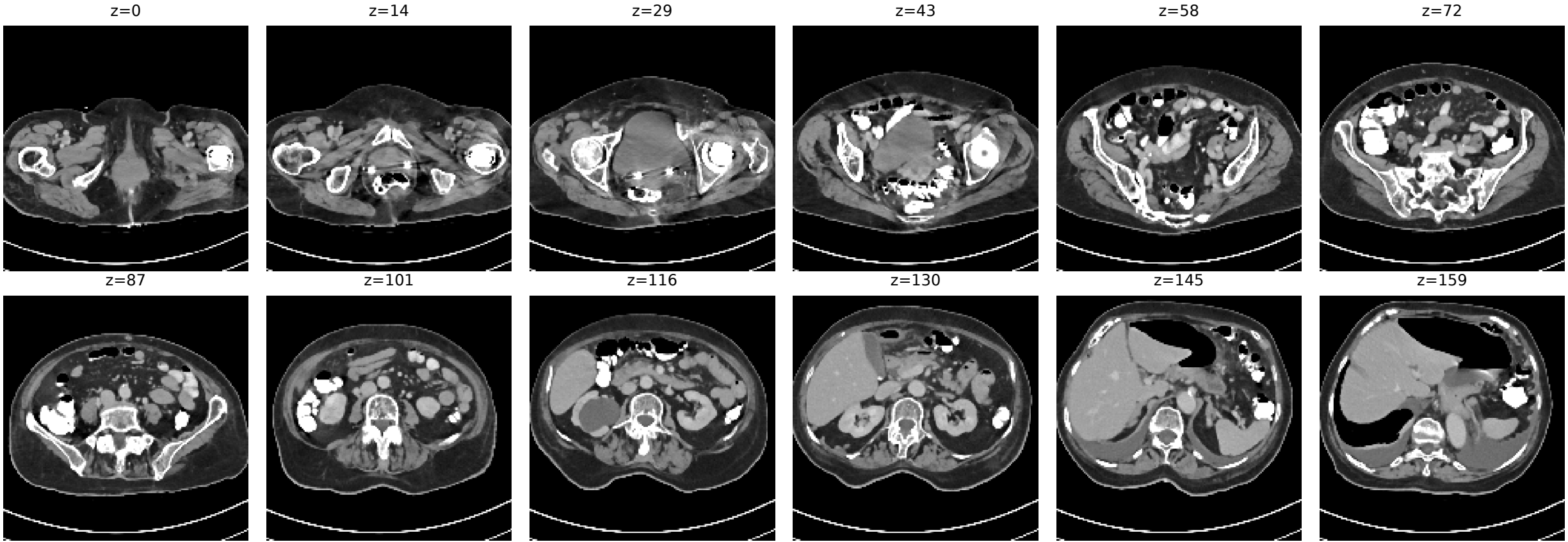
Coronal · model index 126



Sagittal · model index 16



LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R14: model-input axial overview | Perforation → Normal
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R15 | Likely error / uncertain reference

Recorded class: Normal | FlexiCT prediction: Cholezystitis

Radiologist interpretation

Gallstones, chronic wall changes and symptoms improving with antibiotics; no definitive reference diagnosis.

Interpretation of the reread

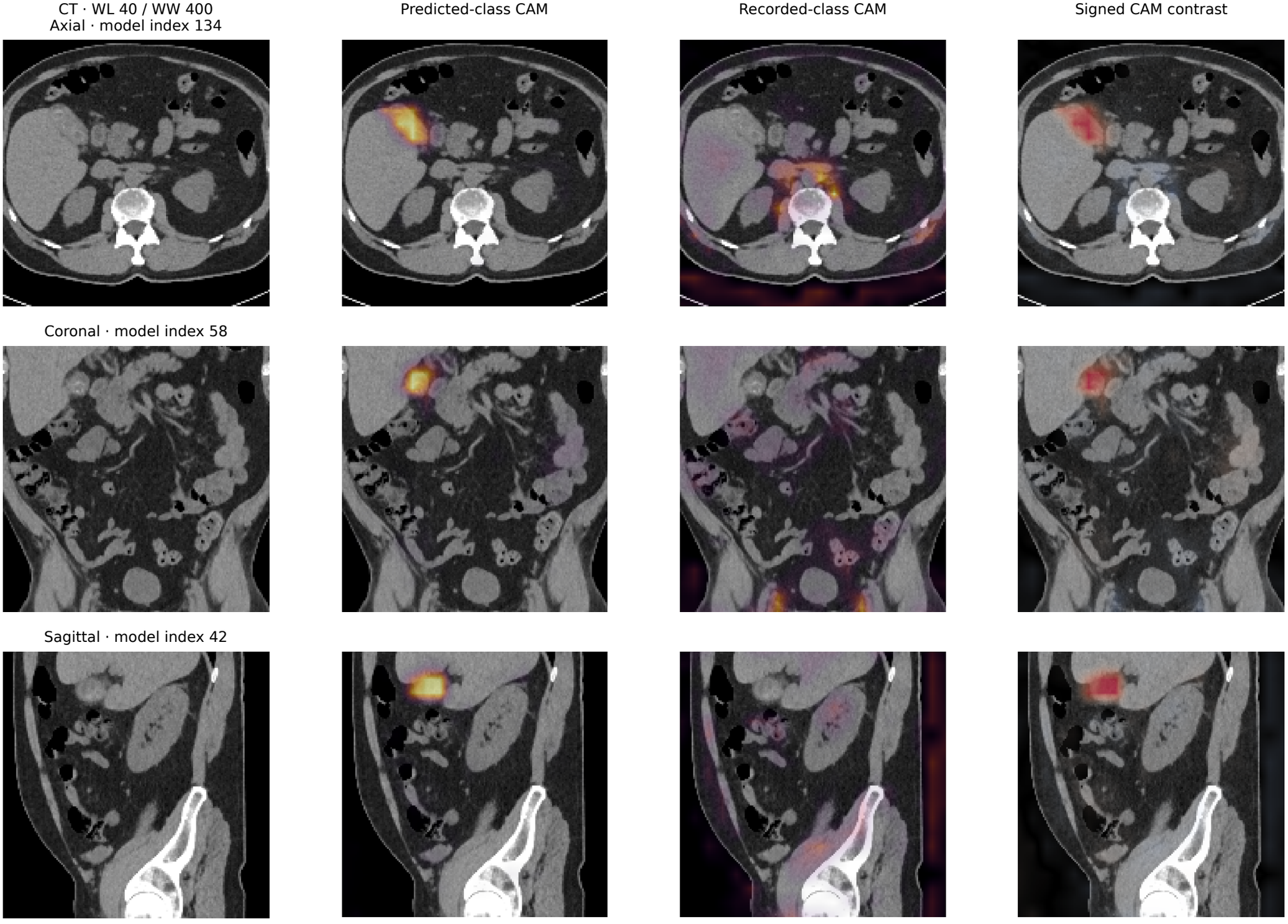
Reviewer considers both labels defensible

Prediction and explanation diagnostics

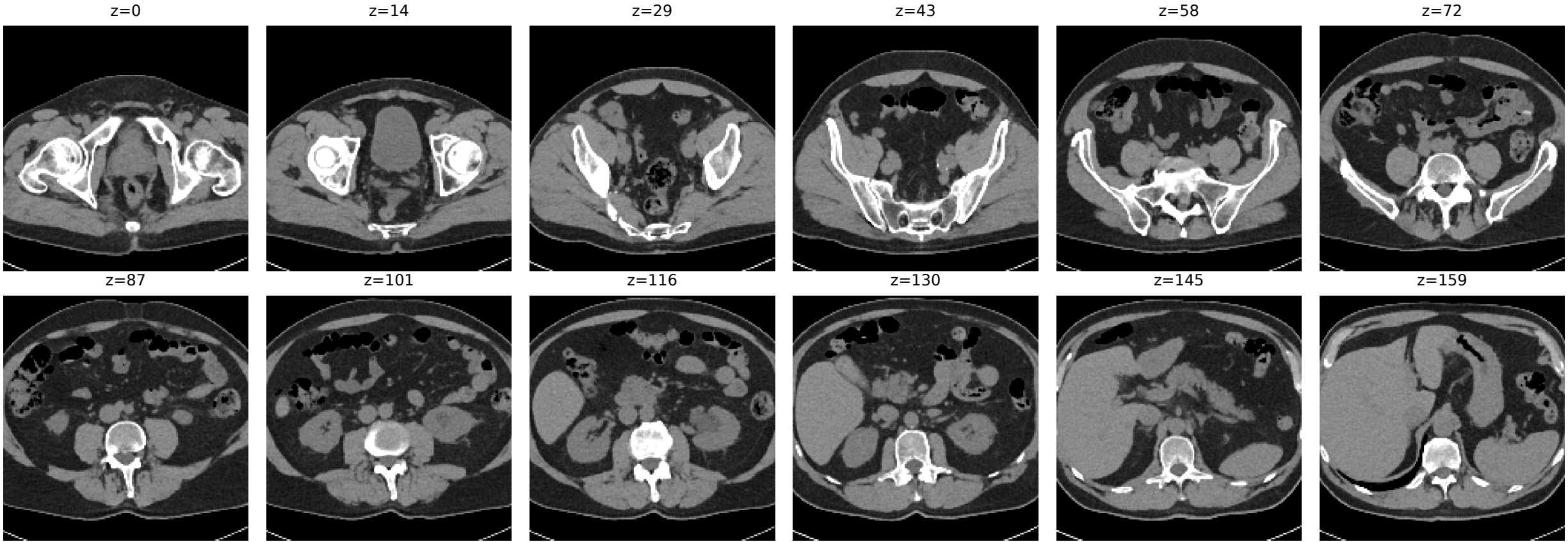
Predicted-class probability	0.7111
Recorded-class probability	0.2859
Fold votes for ensemble prediction	4
Top-mask probability drop	0.7086
Random-mask mean probability drop	0.3857
Positive drop exceeds random	True
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

R15 · Recorded: Normal · Predicted: Cholezystitis
p(predicted)=0.711 · p(recorded)=0.286 · Likely error / uncertain reference



LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R15: model-input axial overview | Normal → Cholezystitis
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R16 | Confirmed model error

Recorded class: Appendicitis | FlexiCT prediction: Normal

Radiologist interpretation

Clear uncomplicated appendicitis; renal transplant changes anatomy.

Interpretation of the reread

Anatomical explanation is a reviewer hypothesis

Prediction and explanation diagnostics

Predicted-class probability	0.6185
Recorded-class probability	0.3724
Fold votes for ensemble prediction	4
Top-mask probability drop	0.0236
Random-mask mean probability drop	-0.1888
Positive drop exceeds random	True
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

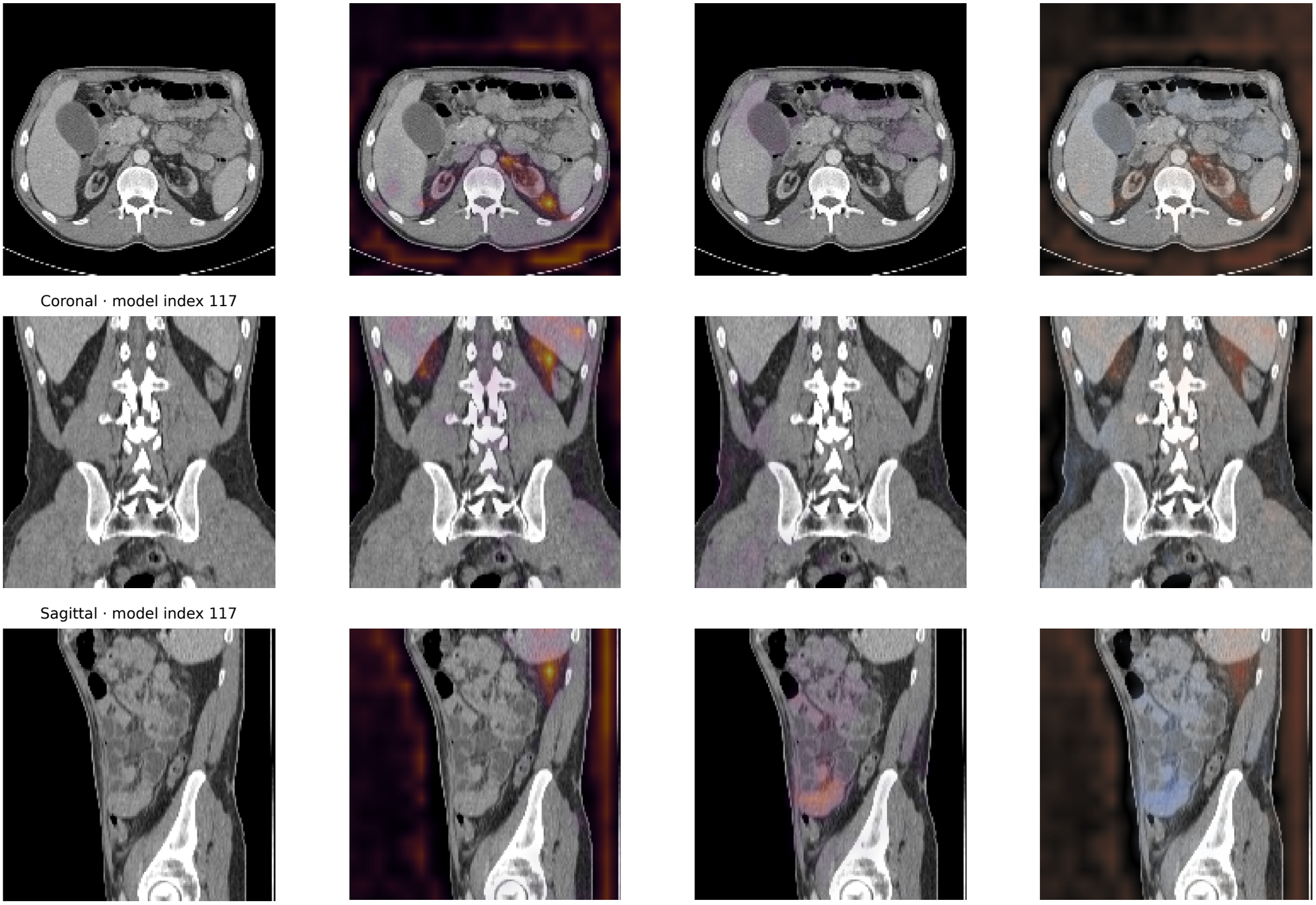
R16 · Recorded: Appendicitis · Predicted: Normal
p(predicted)=0.618 · p(recorded)=0.372 · Confirmed model error

CT · WL 40 / WW 400
Axial · model index 134

Predicted-class CAM

Recorded-class CAM

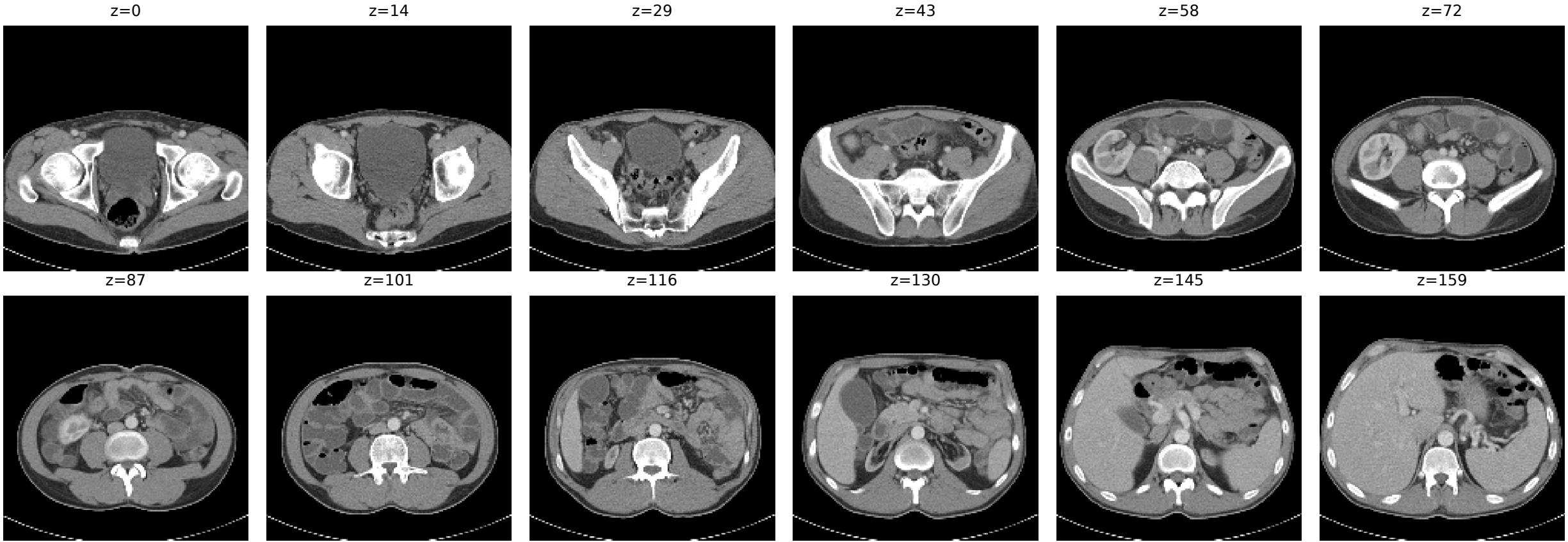
Signed CAM contrast



Coronal · model index 117

Sagittal · model index 117

LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R16: model-input axial overview | Appendicitis → Normal
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R17 | Other disease / proposed exclusion

Recorded class: Appendicitis | FlexiCT prediction: Normal

Radiologist interpretation

Extensive ulcerative colitis with secondary appendiceal enhancement rather than primary appendicitis.

Interpretation of the reread

Reviewer recommends exclusion

Prediction and explanation diagnostics

Predicted-class probability	0.5901
Recorded-class probability	0.0803
Fold votes for ensemble prediction	3
Top-mask probability drop	-0.0851
Random-mask mean probability drop	-0.1236
Positive drop exceeds random	False
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

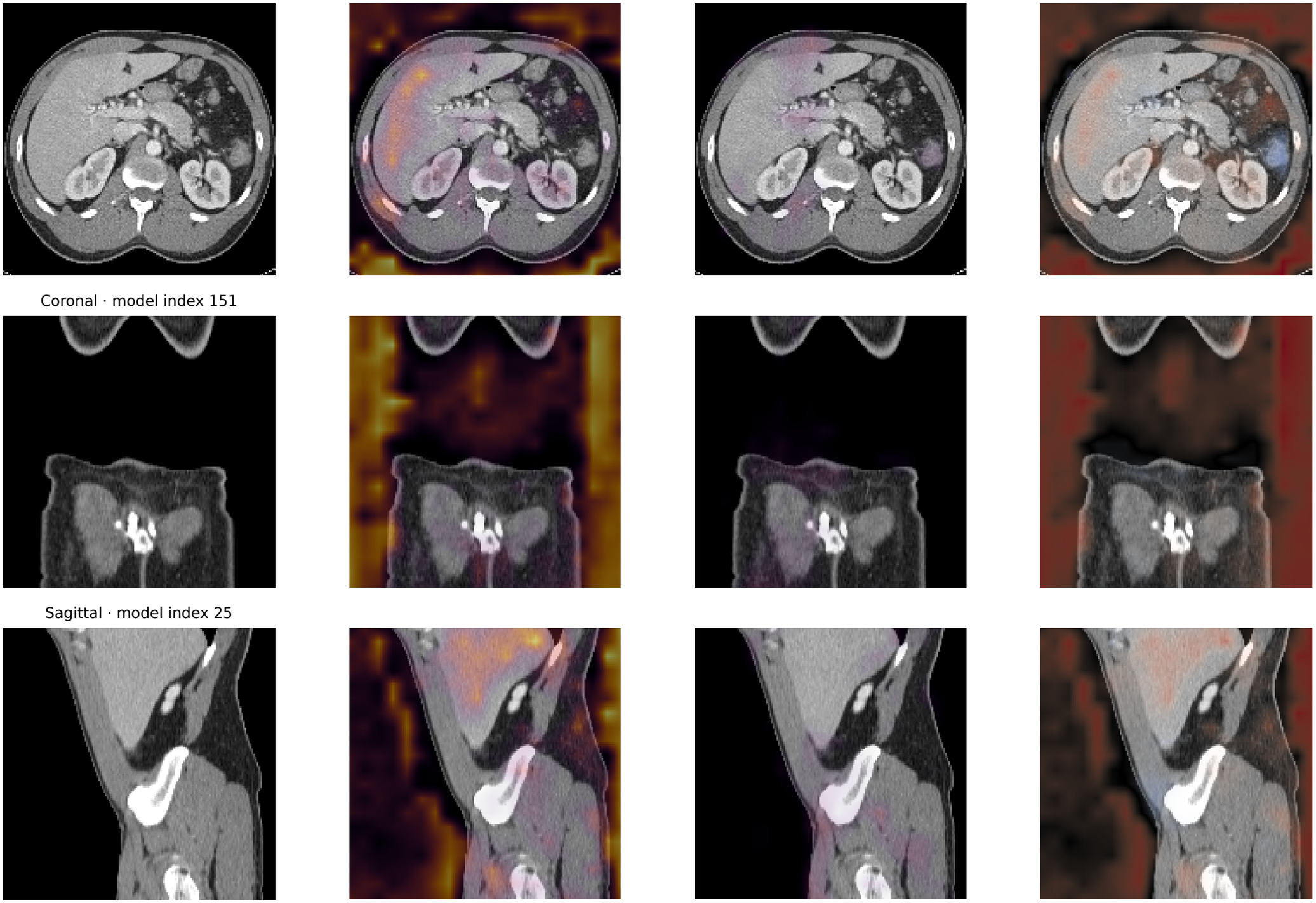
R17 · Recorded: Appendicitis · Predicted: Normal
p(predicted)=0.590 · p(recorded)=0.080 · Other disease / proposed exclusion

CT · WL 40 / WW 400
Axial · model index 142

Predicted-class CAM

Recorded-class CAM

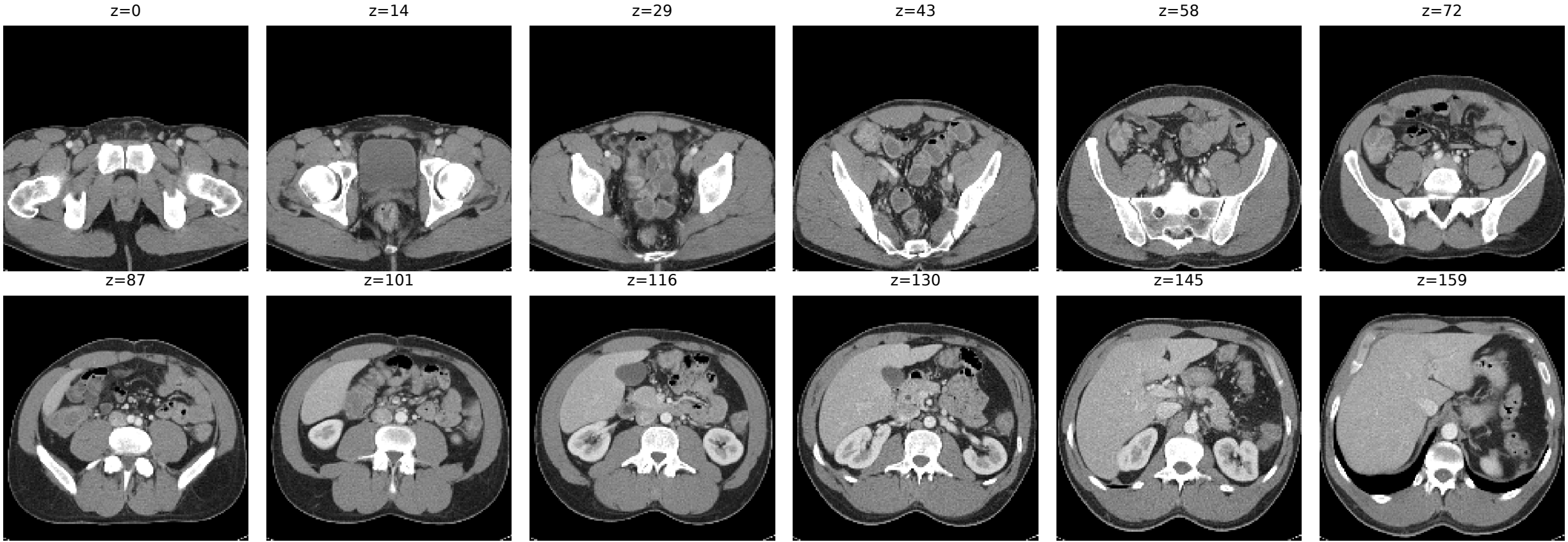
Signed CAM contrast



Coronal · model index 151

Sagittal · model index 25

LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R17: model-input axial overview | Appendicitis → Normal
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R18 | Reference-label discrepancy

Recorded class: Normal | FlexiCT prediction: Sigmoid

Radiologist interpretation

Bacterial colitis with focal sigmoid inflammation and diverticula; diverticulitis considered likely.

Interpretation of the reread

Reread supports model class provisionally

Prediction and explanation diagnostics

Predicted-class probability	0.5591
Recorded-class probability	0.4260
Fold votes for ensemble prediction	3
Top-mask probability drop	0.5555
Random-mask mean probability drop	0.3761
Positive drop exceeds random	True
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

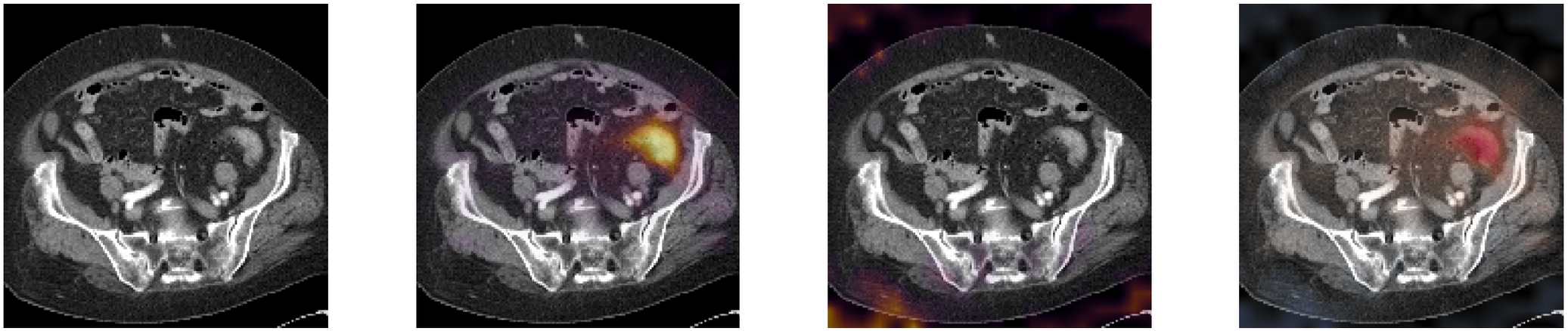
R18 · Recorded: Normal · Predicted: Sigmoid
p(predicted)=0.559 · p(recorded)=0.426 · Reference-label discrepancy

CT · WL 40 / WW 400
Axial · model index 50

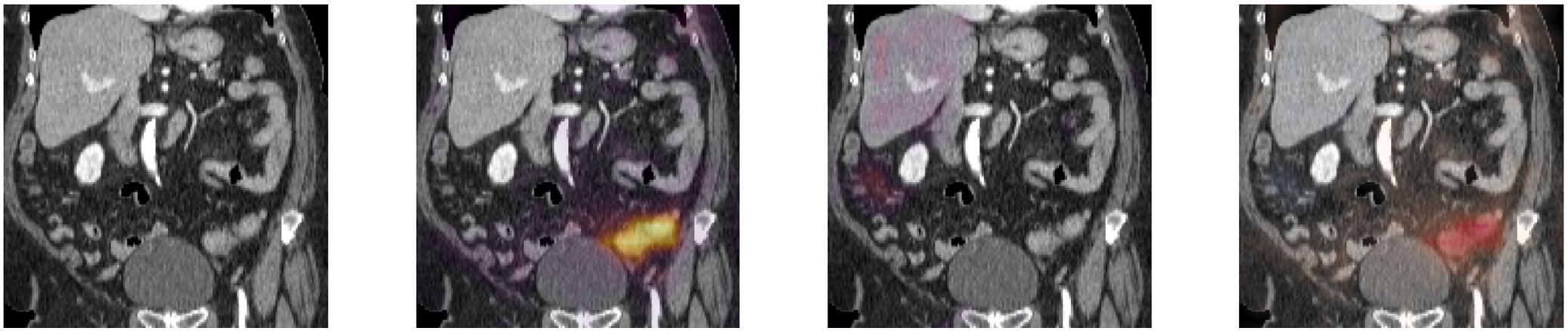
Predicted-class CAM

Recorded-class CAM

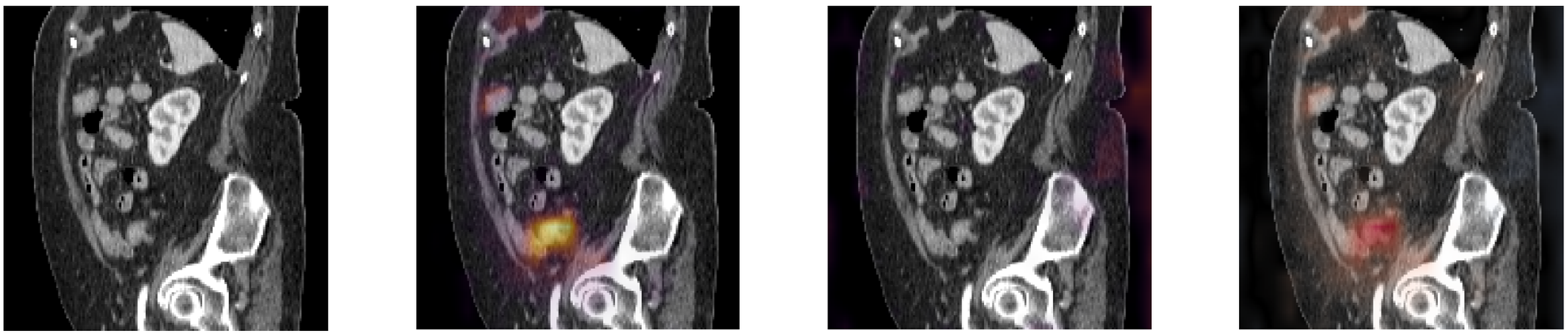
Signed CAM contrast



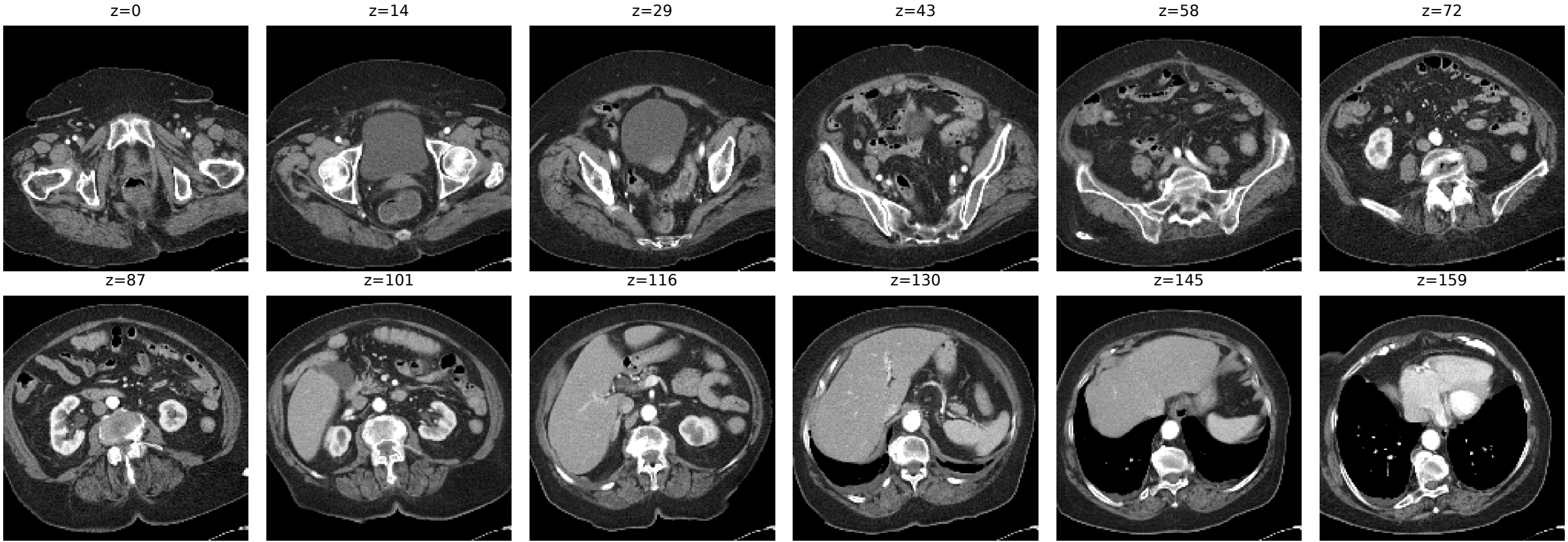
Coronal · model index 67



Sagittal · model index 117



LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R18: model-input axial overview | Normal → Sigmoid
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R19 | Overlapping target diagnoses

Recorded class: Sigmoid | FlexiCT prediction: Perforation

Radiologist interpretation

Sigmoid diverticulitis with perforation.

Interpretation of the reread

Single-label task conflates coexisting diagnoses

Prediction and explanation diagnostics

Predicted-class probability	0.5506
Recorded-class probability	0.4426
Fold votes for ensemble prediction	3
Top-mask probability drop	0.1521
Random-mask mean probability drop	-0.0488
Positive drop exceeds random	True
Linkage	exact_id

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

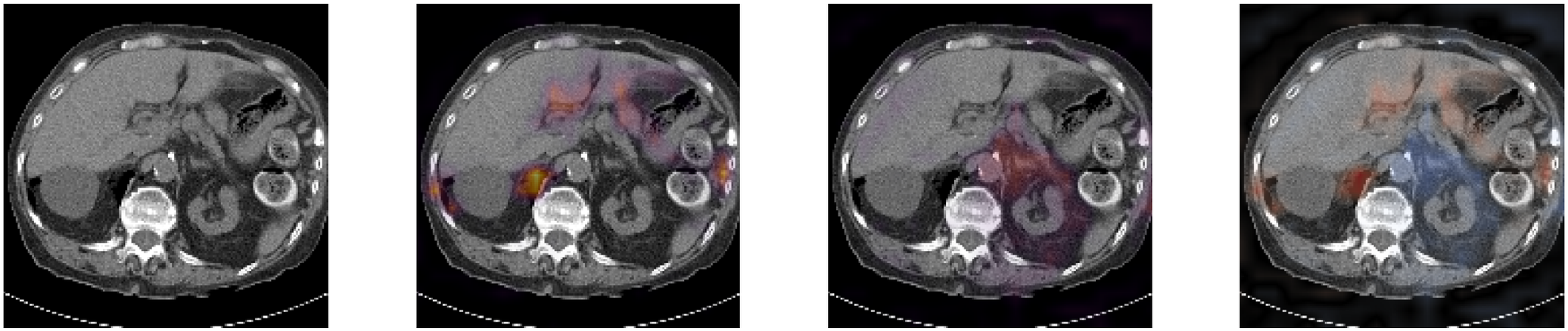
R19 · Recorded: Sigmoid · Predicted: Perforation
p(predicted)=0.551 · p(recorded)=0.443 · Overlapping target diagnoses

CT · WL 40 / WW 400
Axial · model index 150

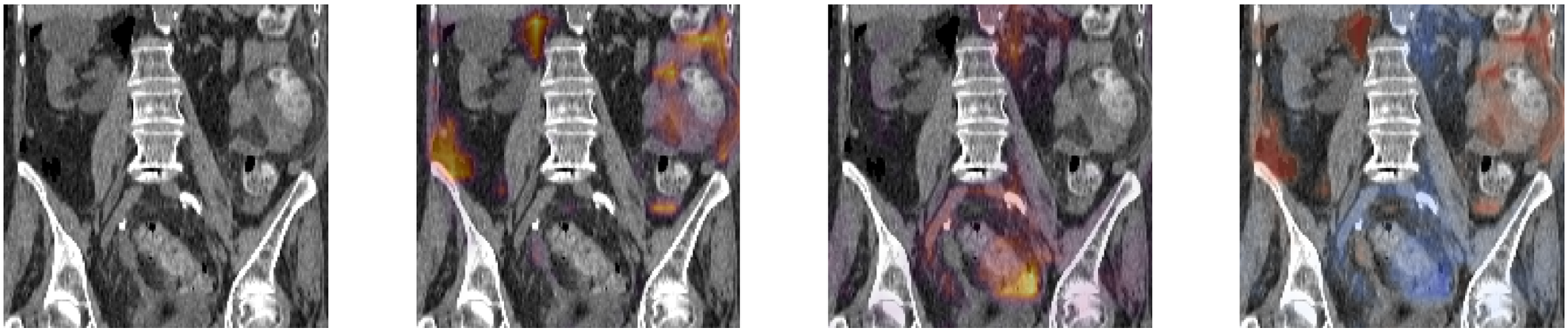
Predicted-class CAM

Recorded-class CAM

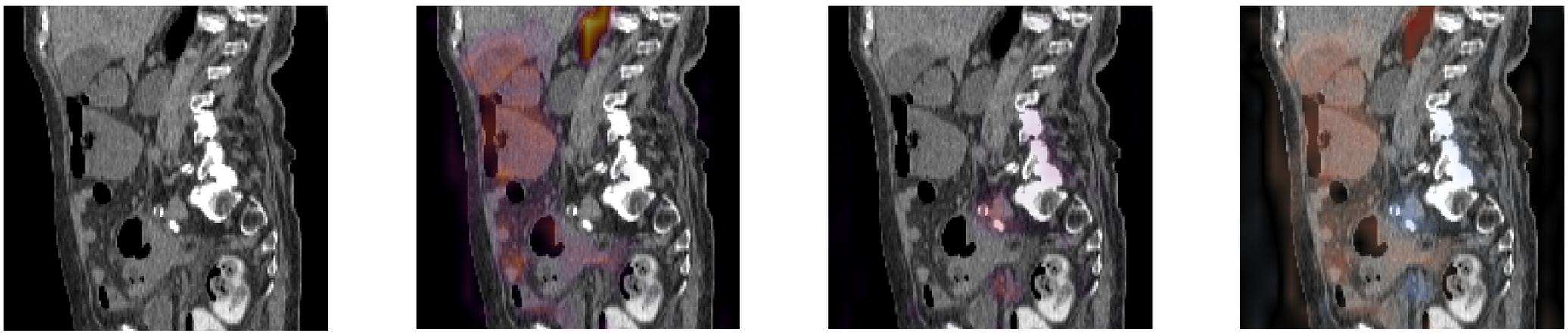
Signed CAM contrast



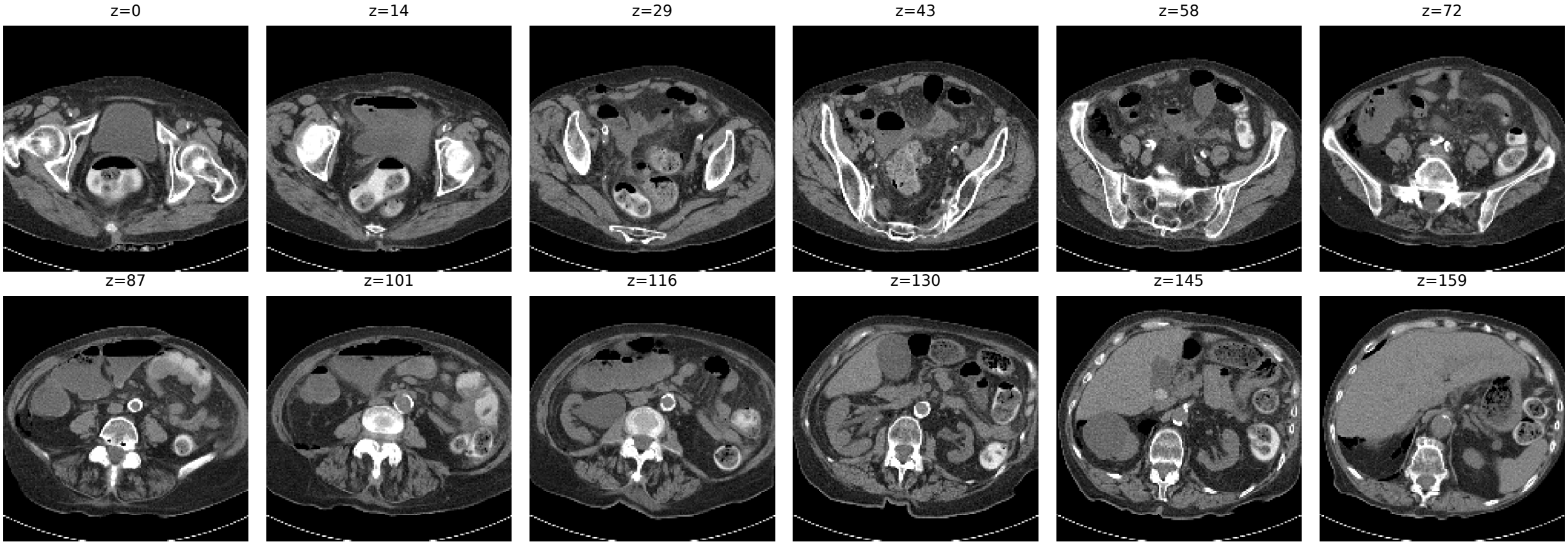
Coronal · model index 84



Sagittal · model index 59



LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R19: model-input axial overview | Sigmoid → Perforation
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159

R20 | Likely error / uncertain reference

Recorded class: Perforation | FlexiCT prediction: Cholezystitis

Radiologist interpretation

Reviewer describes pyloric perforation with minimal free air and gallbladder hydrops mimicking cholecystitis; exclusion or discussion suggested.

Interpretation of the reread

Provisional linkage: original correspondence identifier differs

Prediction and explanation diagnostics

Predicted-class probability	0.3844
Recorded-class probability	0.2677
Fold votes for ensemble prediction	2
Top-mask probability drop	0.3767
Random-mask mean probability drop	0.2683
Positive drop exceeds random	True
Linkage	provisional_user_inference

The following two pages reproduce this case's three-plane attribution figure and CT overview. The recorded reference class is unchanged.

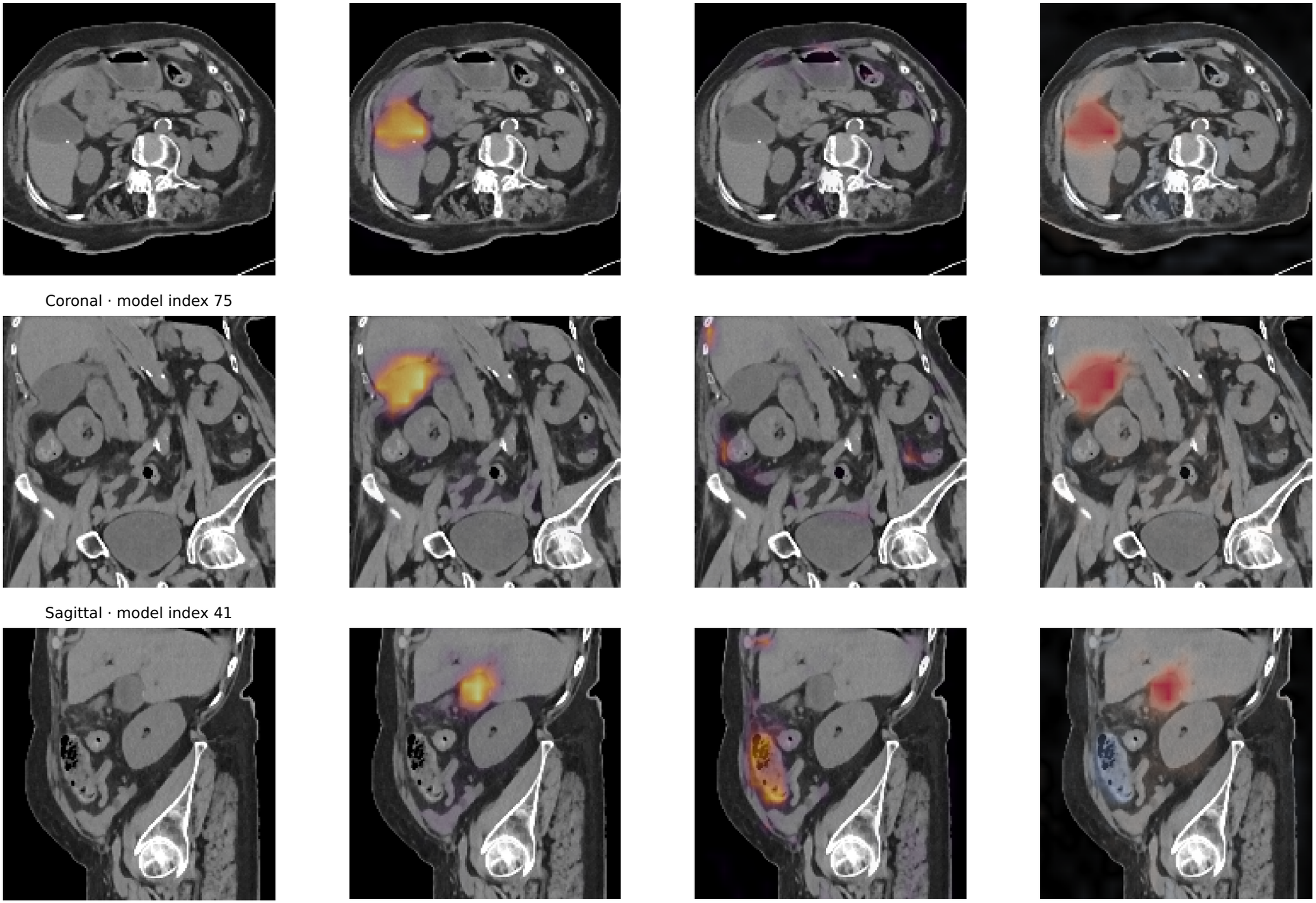
R20 · Recorded: Perforation · Predicted: Cholezystitis
p(predicted)=0.384 · p(recorded)=0.268 · Likely error / uncertain reference
PROVISIONAL COMMENT–CASE LINKAGE

CT · WL 40 / WW 400
Axial · model index 118

Predicted-class CAM

Recorded-class CAM

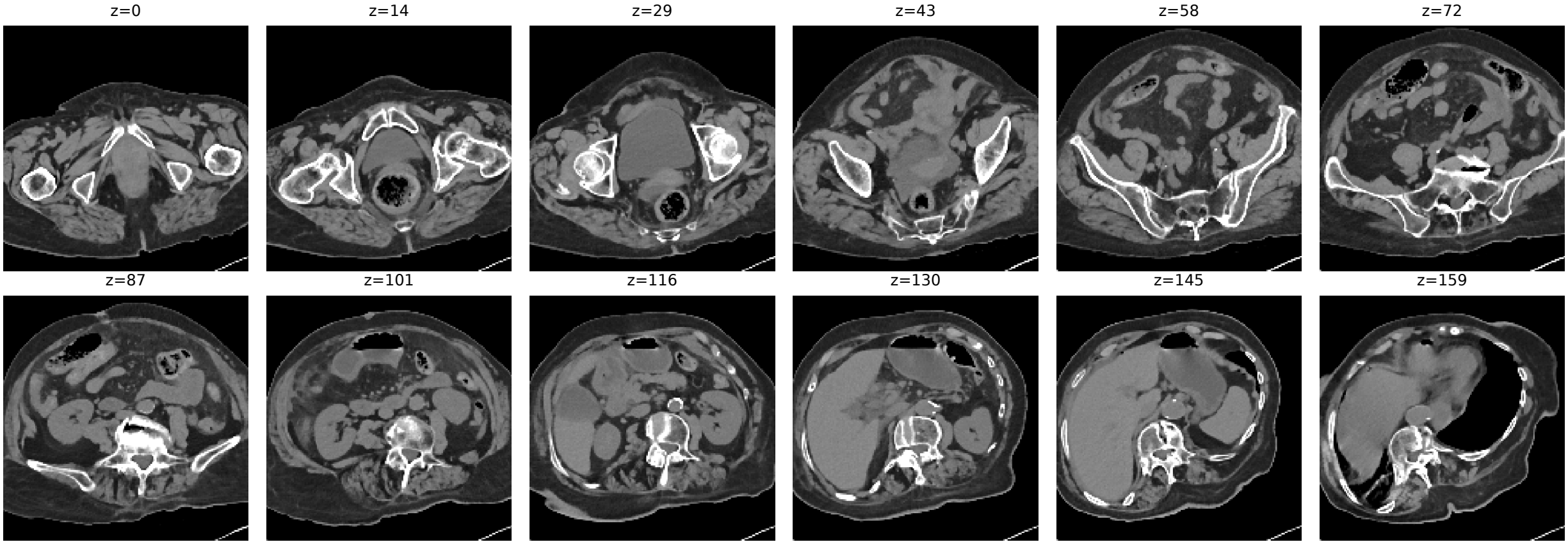
Signed CAM contrast



Coronal · model index 75

Sagittal · model index 41

LPS model space · 2 mm voxels · 16 mm native map grid · planes at predicted-map maximum
Positive scales independent: 0 (transparent) to 1 (yellow). Contrast: blue recorded / red predicted.
Cropped input and qualitative maps; no validated lesion localization.


R20: model-input axial overview | Perforation → Cholezystitis
z=0
z=14
z=29
z=43
z=58
z=72
z=87
z=101
z=116
z=130
z=145
z=159